

15 May 2023

Lucy Letby - Examination-in-chief day 3: Babies F, G, H & I

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MS LUCY LETBY (continued)

Examination-in-chief by MR MYERS (continued)

Monday, 15 May 2023

(10.30 am)

(In the presence of the jury)

MS LUCY LETBY (continued)

Examination-in-chief by MR MYERS (continued)

MR MYERS: Ms Letby, you were last giving evidence on 5 May, we're now the 15th. I just want to bring us up to speed. There have been delays for good reason, delays that are unavoidable, but over this period you've been waiting to continue, haven't you?

LETBY: Yes.

MYERS: All right. And just so everyone can understand the position, you haven't spoken with your legal representatives over the period we've been waiting, have you?

LETBY: No.

MYERS: That of course includes me. Just so we all understand, in the normal course of events, and this is no different, a witness isn't allowed to speak with legal representatives once they start to give evidence. Certainly a defendant doesn't in Ms Letby's position.

So we're picking up really where we finished 10 days ago, aren't we; yes?

LETBY: Yes.

MYERS: All right. So just tune in and get comfortable, if it's at all possible where you're sitting, Ms Letby, to do that, all right? I want you to listen carefully, but we'll take this steadily as we get going again, all right? Are you listening to me?

LETBY: Yes.

MYERS: Keep your voice up and look at the ladies and gentlemen when you're answering questions and we'll take our time with it.

I'm going to bring us back to where we were with [Baby F], which is count 6, ladies and gentlemen, and we were looking at that when we were last here.

[Baby F], of course, was the twin of [Baby E]. All right? Born on 29 June 2015 at 17.54, 29 weeks and 5 days' gestation, and he weighed 1.434 kilograms.

We're not going to go over everything that we looked at when we dealt with [Baby F], but what I am going to do is just remind ourselves, and I'll ask you if you can look at this too, ladies and gentlemen, of that chart with the blood sugars so we know what we're dealing with.

We should know where that is now as we had a practice run at this on the 5th. It should be behind divider 5 in jury bundle 1, so I'm going to ask if you can follow what we're looking at, Ms Letby. I know it's me talking to start with just to bring us back to where we were.

(Pause)

In [Baby F]'s case, Ms Letby, as we all know, from about 1.54 am on 5 August, he had low blood glucose and that continued through until about 19.00 hours that day.

Just to remind us, we'll look at the chart, a sample of blood taken at 17.56 that day returned an insulin reading of 4,657 picomoles per litre and a C-peptide of 169 picomoles per litre. You will remember, and the jury will remember, that that ratio is out. That's far too high insulin for naturally occurring insulin, we're told, and therefore that's what we're looking at.

Let's just look at the chart, Ms Letby, if you just look at that with me, please. Let's look from 5 August to remind ourselves of the figures. At 01.54 in the morning, blood glucose was at 0.8, which as we know is a low reading.

Just dealing with what's on the chart, at 02.55 it was at 2.3, but we know, and we'll return to this, there had been a bolus of dextrose given to [Baby F] in between those two readings.

At 04.02, it was down to 1.9. At 5 o'clock in the morning it was up to 2.9, but again there had been a bolus of dextrose given in between those readings. At 08.09, 1.7, which is low again. 10 o'clock, 1.3.

11.46, 1.4. 12 o'clock, 2.4. At 14.00 it was 1.9 -- these are low -- and at 16.00, 1.9, 18.00, 1.9.

The TPN was stopped at 18.55 and dextrose simply was run through. At 19.00 it was up to 2.5, the blood glucose. At 21.17, 4.1.

So we're going to look at what happens with the bags as far as you can tell us, Ms Letby, over this period.

I should say [Baby F], we established this also, received care from you before this period and also, if you remember, afterwards. You looked after him on other dates, didn't you?

LETBY: Yes.

MYERS: All right. Let's have a look at the bags. I'm going to ask if we can put on the screen, please, tile 17. This is a chart for the TPN bags that we don't have in paper, ladies and gentlemen, but it's where the TPN bags start when we're following them with [Baby F]. If we can open that, thank you.

I'm going to ask if we can take a look at the whole document first so we know what we're dealing with and then we can go and look at the detail.

So this is the sheet we're looking at and it starts with 1 August and if we keep in mind that 5 August is the date that we are focusing on.

Let's have a look at the first entry, if we could, for day 4, please, Mr Murphy. And you take a look, Ms Letby.

Take a moment to have a look there. This is dated 1 August. First of all, can we see your signature anywhere in this section of the form?

LETBY: Yes, for the lipid.

MYERS: For the lipid. Does the mouse work, the one that's just by you there? Do you want to see how you get on with that? Point to where your signature is. That's your signature? Thanks, Ms Letby.

Are you able to tell us in summary what you signed for, what this represents on 1 August?

LETBY: So that is starting a lipid infusion.

MYERS: And we know that because across on the left-hand side it's got "lipid" on that line, hasn't it?

LETBY: Yes.

MYERS: What time did that lipid infusion start that you signed for?

LETBY: 00.20 on the 2nd.

MYERS: So that's what we have on the next to last column on the right-hand side?

LETBY: Yes.

MYERS: And we can see, going right of that, it says "3 August 15, 00.10"; what does that tell us?

LETBY: That's the time that that infusion was taken down and discarded.

MYERS: So would that box be filled in when that infusion is taken down?

LETBY: Yes, only once it's been taken down.

MYERS: So what does that tell us about when it was taken down then when we look at that box?

LETBY: It was taken down at 00.10 hours on the 3rd and that's because lipid only lasts 24 hours.

MYERS: All right. Yours isn't the only signature in the box.

Why is there someone else who signed?

LETBY: Because myself and the other signature are the ones that have drawn up that lipid and commenced it (overspeaking) need two signatures.

MYERS: Right, two signatures. Going left of there, we've got the date, we know that, 2 August, and then a signature.

Who is it that signs the box where it says "prescribed by"?
Whose role --

LETBY: That's the doctor who's written the prescription.

MYERS: Thank you. That lipid was started at 00.20 on 2 August.

Was there already a TPN bag up at that point?

LETBY: Yes, there was.

MYERS: How do we know there was already a TPN bag up at that point?

LETBY: The column above is referencing the TPN and it says a continuing 48-hour bag.

MYERS: Is that your writing?

LETBY: It is, yes.

MYERS: Was that 48-hour bag eventually changed?

LETBY: Yes. So if we look on the end column it was changed at the same time as the lipid syringe on 03/08.

MYERS: Right, at 00.10. Is there a significance in these timings round about midnight when it comes to the changing of bags or syringes?

LETBY: Midnight is the time that we tend to change our fluids.

So that's standard practice for it to be around midnight.

MYERS: So that's why we tend to see times around midnight for this?

LETBY: That's right, yes.

MYERS: Thank you very much. If we move down, please, Mr Murphy, to what's down as day 5, 2 August 2015.

Thank you. We can see days 5 and 6, as they're put here.

Let's look at 2 August. Have you signed for anything on the Babiven line for 2 August?

LETBY: Yes.

MYERS: What have you signed for?

LETBY: I've signed for the Babiven and for the lipid syringe.

MYERS: And somebody else has co-signed with you?

LETBY: Yes.

MYERS: What happened with the TPN, the Babiven? What are you signing to show?

LETBY: I'm signing to show that a new bag of TPN has been commenced on 3/8 at 00/10.

MYERS: Does that follow on from the one that was above it that we saw that finished --

LETBY: Yes.

MYERS: -- on 3 August? So this is continuing?

LETBY: This is a new bag. So it's following -- the bag was taken down in the box we've just looked at and this is the new TPN and new lipid.

MYERS: And that's what those two entries refer to, TPN and then lipid?

LETBY: Yes.

MYERS: So both starting now on 3 August at 00.10?

LETBY: Yes.

MYERS: How long is a TPN bag meant to last for?

LETBY: TPN bags last 48-hourly (sic).

MYERS: And the lipid, is that meant to last a particular time?

LETBY: 24.

MYERS: So that's what's going on on 2 August. Looking below that, someone's entered something in error and scrubbed that out; is that correct?

LETBY: Yes.

MYERS: So the next item we come to is at day 6, which is at tile 147 and we do have this in paper if you want to follow it, ladies and gentlemen. I'll let you know when I suggest we definitely need to go there, but if you want to follow it, it's in paper form in divider 6 of jury bundle, but we're putting it up on the screen. And that's divider 6 of jury bundle 2, but it's on the screen in any event.

Looking at day 6 then, 3 August 2015, we know that on the 2nd, just after midnight, you'd signed for a bag of TPN that was hung up. So what's happening on the TPN line for 3 August?

LETBY: This bag is now 24 hours old which means it doesn't need changing so I'm signing to say it's a continuing bag that's running.

MYERS: So that's your signature where it says "given by"; is that correct?

LETBY: Yes.

MYERS: But you've not actually given anything as it happens at that point?

LETBY: No, that's why it's a single signature because nothing new has been started. It's me confirming that the bag is the same bag that was running the day before.

MYERS: Is that why it says "Continuing 48-hour bag"?

LETBY: Yes.

MYERS: And the column on the right-hand side that says "Time and date finished", what's that telling us?

LETBY: So that's the time that this TPN bag was discontinued.

MYERS: Right, thank you. Are you able to help us with what's happened where it says on the next line down, day 7, the next box down, 4 August 2015, can you help us with what's happened where we see various things but then a line put through it?

LETBY: So that is -- the first prescription has been crossed through and re-prescribed by the doctor because in the top box the baby's receiving milk but in the bottom column it isn't, so the prescription's had to be changed.

MYERS: Can you explain what you mean by that a little more?

You can use the mouse if you need to point things out to us.

LETBY: This signature here (indicating) is myself and another nurse to say that a TPN bag was started on 05/08 at 00.25 and lipid was not required because the baby was receiving enteral feeds, which we can see in the enteral column here. So what we notice in the prescription below is the baby is no longer having milk feeds, therefore lipid is needed to be prescribed and therefore it has had to be crossed through above and rewritten on the next prescription.

MYERS: The one that's crossed out was done on the basis that the baby would receive -- that [Baby F] would receive an enteral milk feed?

LETBY: Yes, and therefore did not require lipid --

MYERS: Right.

LETBY: -- which is why it says "not required", but something has changed and he is now requiring lipid, which means the prescription needs to be rewritten.

MYERS: So what we have at day 7 on the bottom that we can see, without the line through it, is the prescription that was actually followed; is that right?

LETBY: Yes.

MYERS: Let's look at that one. We can see signatures and we can see a time on the right-hand side, the box to the right for the TPN and it says 00.25 -- what does that say, what's the date?

LETBY: The 5th.

MYERS: 5 August 2015. Can you tell us, first of all, what's happened there?

LETBY: A new TPN bag has been started.

MYERS: When was that TPN bag hung?

LETBY: On the 5th at 00.25.

MYERS: Did that replace the one that had been there in fact --

LETBY: It's the same bag. So the TPN bag is the same in both, it's the same bag in both of these prescriptions.

MYERS: Right.

LETBY: It's just that the bottom, this one here (indicating), is now a lipid infusion that's starting at a later time of 3 am.

MYERS: So where we see the time starting at 00.25 for 5 August, that corresponded with the entry at the top --

LETBY: Yes, it does.

MYERS: -- if we just look at the top of this page, for where it says "Time and date finished".

LETBY: Yes.

MYERS: If we just look at the top right-hand box again, can you see there, ladies and gentlemen, that bag was timed as finished at 00.25 on 5 August. So this bag replaced that?

LETBY: Yes.

MYERS: Let's scroll down again to day 7, please. How many nurses were involved in hanging that bag?

LETBY: Two.

MYERS: That's why there's two signatures?

LETBY: That's right.

MYERS: And underneath that, can you tell us what's happening where it's got a reference to lipid?

LETBY: In which one, sorry?

MYERS: On the same -- on 4 August 2015 we can see a line for lipid. Could you just tell us what that tells us when you --

LETBY: The one that's crossed through?

MYERS: No, the one for day 7 that isn't crossed through.

LETBY: So that is two signatures to start a lipid syringe.

It's not my signature but it's two members of staff who have started a lipid infusion at 03.00.

MYERS: I'm going to ask if we can go next -- pause for a moment because, of course, we know from the chart we looked at earlier that at 01.54, so an hour and 25 minutes after this bag had been hung, [Baby F]'s blood glucose had dropped to 0.8.

LETBY: Yes.

MYERS: You're aware of that, Ms Letby?

LETBY: Yes.

MYERS: When you gave evidence last time I asked you about what's been said with regard to the insulin and since we're dealing with it now I'll ask you again: is there anything that you did or that you know of that accounts for a drop in blood sugar at that time?

LETBY: No.

MYERS: When we looked at the chart we saw that between 1.54 and 2.55, if you have the chart you can refer back to it, ladies and gentlemen, there was an increase in the blood sugar in fact. Let's just look at how that happens.

Could we put up tile 139, please, Mr Murphy?

Scroll down, please, to 5 August, down towards the bottom. We can see at 2.55 the blood sugar has increased to 2.3. Can you see that, Ms Letby?

LETBY: Yes.

MYERS: I'm going to show you another tile and then ask you a question about it. That's the change in blood sugar.

Can we look at tile 191 next, which is a prescription.

If we look four lines down, can you see we have an entry at 2.05 on 5 August?

LETBY: Yes.

MYERS: Can you tell us what's happened there, please?

LETBY: That is a 10% dextrose bolus that has been given, so 3ml of dextrose.

MYERS: We know the blood sugar has risen by 2.55 --

LETBY: Yes.

MYERS: -- when we looked at the blood glucose.

LETBY: Yes.

MYERS: We know, also recorded, that increase of blood glucose between 4.02 and 5 o'clock in the morning from 1.9 to 2.9. Looking at this chart, can you see anything that could account for why it is that the blood glucose increases for [Baby F] between 4.02 and 5 o'clock that morning?

LETBY: Yes. So he's received another dextrose bolus at 04.20.

MYERS: Could you move the cursor, if you would, so we can see where you're talking about? All right. Thank you. So are you identifying the line for us, Ms Letby? It'll respond now or should do.

LETBY: No, it's not.

(Pause)

Here at 04.20 (indicating).

MYERS: Right. So that's a dextrose bolus that's received by [Baby F] between the readings at 4.02 and 5 o'clock?

LETBY: Yes.

MYERS: When the blood glucose increases?

LETBY: Yes.

MYERS: Had you been working the night shift on this particular day or night?

LETBY: Yes.

MYERS: So what time would your shift had ended?

LETBY: 8 am.

MYERS: Right. Would you have concluded on the unit at that time?

LETBY: Around that time, depending on when I finished handover, but yes, around that time.

MYERS: Just keeping that in mind, can we please go next to tile 261, which we also have in paper, ladies and gentlemen, at page 3144 for anyone who's following it in paper.

I'd like us to look at the entry for day 8, the top box, for 5 August 2015. Just make sure we can all see that. Were you responsible for the care of [Baby F] at this time when we come to 5 August?

LETBY: No, I wasn't.

MYERS: By "this time" I'm looking at an entry that says 12.00; can you see that?

LETBY: Yes.

MYERS: We can all see that. Your shift had concluded that morning?

LETBY: That's right.

MYERS: Looking at what we can actually see for the TPN bag, which is the top line that says "Babiven", can you tell us what has happened at or about 12.00 on this particular day?

LETBY: Yes, so at midday on the 5th a new bag of TPN and lipid has been prescribed and signed for by two nurses.

MYERS: Right. So the two nurses are in the column headed "Given by"; is that right?

LETBY: Yes.

MYERS: That's not your signature on any of them?

LETBY: No.

MYERS: And the time that this was started?

LETBY: That's 12 midday on the 5th.

MYERS: It's got in brackets something that says "new long line"?

LETBY: Yes.

MYERS: By way of reminding us all, the ladies and gentlemen of the jury may recall, than [Baby F]'s line had been identified as tissing during that morning.

LETBY: Yes.

MYERS: This is just reminding us of the evidence. The TPN was stopped for a period and Nurse Shelley Tomlins told us how the line and the bag were changed.

LETBY: Yes.

MYERS: If a line tissues and it's necessary to change the bag, what would you change as part of that procedure?

LETBY: Standard practice is that everything would be discarded and you would start again with everything: new fluids, lines, everything would be new.

MYERS: And you say that's standard, why is that standard practice?

LETBY: Because the TPN is sterile, so the moment that you disconnect it from a line or stop it running, it's therefore no longer sterile. So practice is that once you've broken that line, you've disconnected it or you have to stop it, you would discard that and start again.

MYERS: We've heard the evidence about how that happened from the nurses dealing with this.

LETBY: Yes.

MYERS: If we just look for a moment -- and I'll ask Mr Murphy to take us to tile 200 -- at the entry on the left-hand side for 11.00 or thereabouts. We can see the timing and see what's been signed for down at the bottom in the text.

It's got here a signature at the bottom of "SAT".

LETBY: Yes.

MYERS: Do you know whose initials they are?

LETBY: That's Shelley Tomlins.

MYERS: Shelley Tomlins?

LETBY: Yes.

MYERS: And what has she signed that has happened at this time?

LETBY: A new long line has been inserted into [Baby F] at this point.

MYERS: And that's together with the change of TPN?

LETBY: Yes.

MYERS: We know that even after that, the blood glucose levels remain low?

LETBY: Yes.

MYERS: And they remain low until about 7 o'clock that evening.

Do you know why they remain low even after there has been a line and bag change?

LETBY: No.

MYERS: But at this point is this the same bag that you had hung up at midnight or just after midnight that morning?

LETBY: No, it isn't.

MYERS: And is it the same line?

LETBY: It's not, no.

MYERS: We can take that down, Mr Murphy, thank you.

We've seen how on a later date or dates you continued to care for [Baby F] when you were back on the unit --

LETBY: Yes.

MYERS: -- is that correct? And no issues are identified then --

LETBY: No.

MYERS: -- that you're asked for account for?

I just want to ask you something about Facebook searches at this point. We saw from the schedule that between June 2015 and June 2016, there are 2,381 Facebook searches conducted.

LETBY: Yes.

MYERS: Just to remind us, you'd looked for [Mother of Babies E & F] nine times between 6 August 2015 and 10 January 2016.

LETBY: Yes.

MYERS: And [Father of Babies E & F] on one occasion in October 2015. Why did you search for [Mother of Babies E & F] or put her details into Facebook?

LETBY: Searching people on Facebook is something I would do.

Quite often [Mother of Babies E & F] was on my mind following [Baby E] and [Baby F]. If somebody came into my mind, that's what I would do, I'd just search their name.

MYERS: Does that just go for people at work or does it apply to people elsewhere?

LETBY: No, it could be anyone.

MYERS: And does it apply just to babies on this indictment?

LETBY: No.

MYERS: Is there anything unusual in you looking at someone who's on your mind on Facebook more than once?

LETBY: No, that's a normal pattern of behaviour for me.

MYERS: We've been told about a card that was sent by the [Family of Babies E & F] to the unit.

LETBY: Yes.

MYERS: I'll remind us what the agreed facts said, ladies and gentlemen, so you don't have to turn there unless you want to. If you want to make a note of the agreed facts, it was agreed facts 39 and 40, and I will just remind you of what they were, Ms Letby.

Images of the thank-you card from the [Family of Babies E & F] were recovered from the images on the handset -- on your phone handset.

LETBY: Yes.

MYERS: I'm not putting all the figures in. And further analysis of the data from those images establishes they were taken at 3.40 am on 20 November 2015.

LETBY: Yes.

MYERS: And the coordinates indicate that was taken in a location in the south corner of the Women and Children's Building at the Countess of Chester Hospital.

Right.

Why did you take a photograph of a thank-you card from the [Family of Babies E & F]?

LETBY: It was something I wanted to remember. I quite often take photographs of cards that I've been sent or that I receive.

MYERS: Is there anything unusual in you doing that?

LETBY: No.

MYERS: I'll come back to that when we look at [Baby I] in fact. As for the time this was taken, I wonder whether we could just put up the shift pattern for page 33225, which is in jury bundle 2, divider 23, if you want to see the shift pattern in paper, but we can put it on the screens. It's page J33225.

This image was taken at 3.40 in the morning on 20 November. This shows us for November 2015 your shift pattern. What were you rostered to be doing on 20 November?

LETBY: I was on a run of night shifts at that time.

MYERS: When had they started?

LETBY: On the 19th.

MYERS: So round about the time this photograph was taken, in the early hours of the morning, where would you have been?

LETBY: At work on the unit.

MYERS: Do you recall where the card was when you took the photograph of it?

LETBY: Yes, it was on the nurses' station at work.

MYERS: And that's where you took the photo?

LETBY: It is, yes.

MYERS: Whilst you were at work on that shift?

LETBY: Yes.

MYERS: Which is why we have the time 3.40 in the morning --

LETBY: Yes.

MYERS: -- and the location in the hospital?

LETBY: Yes.

MYERS: All right. And nothing unusual for you to take a photo of card that you've taken or received; is that what you're saying?

LETBY: No.

MYERS: I'm going to return to that a little later.

Those are questions that I had for you with regard to [Baby F], Ms Letby. As I've said before, I won't repeat myself each time, I appreciate moving from one baby to the next might seem a bit inelegant, but it's not meant to be insensitive, but we do need to move on and look at the next charge that we are dealing with.

Are you all right with that?

LETBY: Yes.

MYERS: We're going to turn then to counts 7, 8 and 9 on this indictment. These are the counts, ladies and gentlemen, relating to [Baby G]. Same process: we'll reacquaint ourselves with some of the details, the basic details, and then look at the events that relate to Ms Letby.

I'm going to summarise, because there are a number of counts, but [Baby G] was born on 31 May 2015 at Arrowe Park -- in fact, let's put up tile 1 just to see the details relating to that.

We'll go into that but we don't need to go beyond the face of the tile. 31 May 2015 at Arrowe Park Hospital at 23.57. The events which we're going to be looking at more closely are in September of 2015.

LETBY: Yes.

MYERS: We will recall that [Baby G] had quite a long course in hospital, didn't she?

LETBY: Yes.

MYERS: Born on 31 May, so some months before the events we're looking at, 23 plus 6 weeks' gestation. What's the normal period for gestation if events run in a standard way?

LETBY: 40 weeks.

MYERS: So this is extremely preterm?

LETBY: It is, yes.

MYERS: 535 grams. Spontaneous delivery. We know in the early part of her life [Baby G] was cared for at Arrowe Park. I'm going to return to that, just to refresh our minds about the dates.

On 13 August, [Baby G] is transferred from Arrowe Park to the Countess of Chester; that's on tile 3.

That's about 2.5 months after her birth; is that correct?

LETBY: Yes.

MYERS: Count 7, which is the next count we come to, is on 7 September 2015, 2.15 in the morning. That involved a projectile vomit and desaturation after feeding, followed by apnoea and then further desaturations and problems with intubation and ventilation. We'll come back to these in a little more detail, but just so we can keep, at the start, an outline of what we're dealing with.

Count 7 is 7 September at 2.15 am and begins with a projectile vomit after feeding.

8 September, tile 442, please. [Baby G] was transferred to Arrowe Park Hospital, so following that event, count 7, she's

transferred to Arrowe Park and she remained there until 16 September.

The next event we deal with is count 8 and that relates to 21 September 2015. That took place around 10 o'clock in the morning and relates to what are described as two projectile vomits and then a self-resolving apnoea and a desaturation.

LETBY: Yes.

MYERS: We'll come to the detail of that when we work through these in a little more detail.

You were caring for [Baby G] at that time; is that correct?

LETBY: Yes, I was.

MYERS: Then count 9, and the third of the events on the indictment with [Baby G], is that same day, 21 September 2015, around 15.27 in the afternoon, and this is the event that involved [Baby G] having been behind a screen with a monitor off --

LETBY: Yes.

MYERS: -- at which time there was an apnoea, a desaturation and bradycardia.

LETBY: Yes.

MYERS: And who found her behind that screen?

LETBY: I did.

MYERS: What did you do when you found her?

LETBY: I got help immediately.

MYERS: Those are the three events we're dealing with: 7 September and two on 21 September.

Before we go through --

MR JUSTICE GOSS: Sorry, 8 September.

MR MYERS: 7 September is the first event.

MR JUSTICE GOSS: You said the 8th -- sorry, I beg your pardon: 7 September, count 7.

MR MYERS: Count 8 is 21 September, 10 in the morning.

Count 9 is 21 September, 15.27.

MR JUSTICE GOSS: Sorry. I was misreading my note. It was 8 September that she went back to Arrowe Park Hospital.

Forgive me. My fault.

MR MYERS: There are a lot of dates.

We're going to turn to your evidence in a moment, but just by way of reminding all of us, I won't go into the detail, when Dr Bohin gave evidence we looked at issues, medical issues, concerning [Baby G] while she had been at Arrowe Park Hospital between 2 June and 4 August and a series of events we went through in Dr Bohin's evidence. Do you recall seeing that?

LETBY: Yes.

MYERS: I'm not going to list them all now, that would take a long time, we might put that into a format which can be put in writing, it's the easiest way of following it, but there were a series of desaturations, distensions and secretions over that period.

LETBY: Yes.

MYERS: That was before her transfer, first transfer, to the Countess of Chester Hospital --

LETBY: Yes.

MYERS: -- and in that 2.5 months before the first event.

LETBY: Yes.

MYERS: All right, thank you.

Do you have any general recollection of [Baby G], Ms Letby?

LETBY: Yes.

MYERS: What is that general recollection of her?

LETBY: [Baby G] was a baby that was with us for a long period of time and she stood out as a baby that had quite complex needs and was a very premature baby, which we don't often care for at the Countess, so we all got to know [Baby G] and her family quite well.

MYERS: When you say complex needs, are you able to describe the sort of things you mean by that or are is that just a general expression you are applying there?

LETBY: It's a general expression but a baby born at that gestation inevitably comes with a lot of medical and nursing needs.

MYERS: We're looking at three particular events over 2 days on this indictment. Did you care for [Baby G] on more than that?

LETBY: Yes.

MYERS: Could you put a figure on how many times you cared for [Baby G]?

LETBY: No, but it would be many times.

MYERS: Well, let's go to 7 September then, please, which is count 7. I'm going to ask Mr Murphy just to assist us by putting up the layout for that event, which is at tile 47, please, Mr Murphy. If we just go into that.

The shift starts on 6 September at 19.30. Where were you during the course of this shift, where were you rostered to be?

LETBY: In nursery 1.

MYERS: Where was [Baby G] during this?

LETBY: [Baby G] was in nursery 2.

MYERS: Who was looking after [Baby G]?

LETBY: [Nurse E].

MYERS: Do you remember who the shift leader was for this particular shift?

LETBY: I believe it was Ailsa Simpson -- that's from memory, though.

MYERS: Thank you. If we roll up, we can see there, shift leader -- well, Ailsa Simpson and Christopher Booth it seems.

LETBY: I recall Ailsa, but they may well have shared the role.

MYERS: In terms of nursing staff, five on that evening, including Lisa Walker?

LETBY: Yes.

MYERS: Scroll back down, please, to the actual image. Do you recall Ailsa Simpson being in nursery 1 at points of that evening?

LETBY: Yes.

MYERS: Do you recall being with Ailsa Simpson at any point that evening?

LETBY: Yes.

MYERS: Any point in particular when you recall being with her?

LETBY: I recall being with Ailsa just prior to [Baby G] vomiting.

MYERS: Right. Before we look at the notes, given that you can recall that, can you just describe to us in your own way what happened? We're talking round about 2 through to 2.15 in the morning. Can you describe to us, before we come to the notes, what it was that took place with [Baby G]?

LETBY: My memory is that myself and Ailsa were sat at the nurses' station, I can't remember exactly what we were both doing, but we'd been there for a few minutes at least. Then we heard [Baby G]'s monitor alarming in nursery 2, she was the only baby in the nursery so we knew it was [Baby G], and we also heard quite a loud retching noise, which we thought was very unusual, so we both went very quickly to [Baby G] in nursery 2.

MYERS: So you'd been with Ailsa Simpson?

LETBY: Yes.

MYERS: Where was that that you'd been with her?

LETBY: At the nurses' desk.

MYERS: In fact, looking at the plan we have here, the nursing desk -- can you use the cursor to show us where that would be? I think we know -- oh, all right. Can you see that, ladies and gentlemen?

LETBY: It's just -- it's right opposite nursery 2.

MYERS: So opposite nursery 2?

LETBY: Yes.

MYERS: And your attention, you say, was caught by two things?

LETBY: Yes.

MYERS: What were they again?

LETBY: It was the monitor alarming and also we heard a retching sound.

MYERS: What did you do after you heard that?

LETBY: We both immediately went in to [Baby G] and found her vomiting and struggling to breathe.

MYERS: Was there anybody with her when you went in there?

LETBY: No.

MYERS: Let's have a look at the notes and then we'll go over these details. Let's start, please, if we could, with the notes of [Nurse E], who was the designated nurse for [Baby G], and that's at tile 137.

Looking at the top left entry -- and scroll up, if we could, please, thank you very much. So these are the notes by [Nurse E] made at 7.49, so in the morning at the end of the shift on 7 September. I'm going to identify the notes generally and go then to the specific part that we're looking at. She identifies that she's writing these in relation to care taken over at 19.45.

She describes the various checks and activities with [Baby G]. I'm going to pick this up about halfway down it where it says:

"Fortifier and Gaviscon via alternative bottle/NGT."

Can you see that?

LETBY: Yes.

MYERS: We can all see that:

"Abdomen full but soft with no discolouration. Aspirates minimal. Partial digested milk. PU [passed urine] and BO ++."

Those are standard abbreviations?

LETBY: Yes, they are.

MYERS: And BO is?

LETBY: Bowels opened.

MYERS: "Short period of straining/uncomfortable at start of night when having cuddles with dad. Dr Ventress aware. Usual for [Baby G]. Then had large bowel movement. Dr Ventress not wanting blood gas overnight, to wait for weekly bloods. Nurse L Letby taken over care following vomit/apnoeic episode after 02.00 feed. Her notes to follow."

Let's have a look at the feed, which is at tile 75, please.

This is the feed we're dealing with. Did you make that entry, Ms Letby?

LETBY: No.

MYERS: Can you help us with who did make that entry?

LETBY: [Nurse E].

MYERS: So this is 02.00. What's been given?

LETBY: [Baby G] has been given 45ml of expressed breast milk with fortifier and Gaviscon added and that's via the nasogastric tube.

MYERS: What's Gaviscon given for?

LETBY: It's given for reflux.

MYERS: What does that mean?

LETBY: So very commonly, premature babies can bring up their milk, so they'll vomit after feeds, will be very uncomfortable after feeds.

MYERS: So this is expressed breast milk with fortifier and Gaviscon. How much is being given?

LETBY: 45ml.

MYERS: And the route?

LETBY: Nasogastric tube.

MYERS: In general, from your experience, how long might it take for 45ml to be given via the nasogastric tube?

LETBY: That could take sort of around 10 minutes or so, depending on -- maybe longer because it's got fortifier and Gaviscon within it, which makes the milk thicker.

MYERS: All right. And under "Vomit/asp", what's written there?

LETBY: That says pH 4., so that's the acidity test that [Nurse E] has done prior to the feed.

MYERS: Is there any indication from what we see here that the stomach contents had been aspirated, in other words emptied out, before the 02.00 feed?

LETBY: No.

MYERS: Does a pH of 4 mean that the stomach is empty of milk when that pH was taken?

LETBY: No.

MYERS: Can you get a pH of 4 and there be milk in the tummy?

LETBY: Yes, you can -- and that is all [Nurse E] is looking for at that time, to see that the tube is in the right place.

MYERS: Just explain to us again, what's the link between the tube being in the right place and the pH?

LETBY: So if you've got an acid reaction it means the NG tube is in the stomach as opposed to being somewhere else in the body.

MYERS: So that's what's being checked in your opinion?

LETBY: Yes.

MYERS: And indeed we've had the evidence from [Nurse E] on this.

That's the feed and we've heard that care was transferred to you, so I'm just going to go to your notes at tile 146.

It's on the right-hand side, the lower part, please, Mr Murphy.

I'm not reading all of it, I'm reading the relevant part to what we're dealing with. Just enlarge the first paragraph, please. These notes are written at 8.57, so this is the following morning.

LETBY: Yes.

MYERS: "Written in retrospect for care given from 02.00 to present. [Baby G] had large projectile milky vomit at 02.15. Continued to vomit ++. 45ml of milk obtained from nasogastric tube with air ++. Abdomen noted to be distended and discoloured. Colour improved few minutes after aspirating tube. Remained distended but soft. Reg Ventress asked to review: to go nil by mouth by IV fluids. Doctor called to theatre. At approximately

03.15, [Baby G] had profound desaturation to 20%, marked colour loss with apnoea, bradycardia to 50s. Neopuffed in 100% oxygen. Observations improved but [Baby G] remained apnoeic. Doctors arrived. Intermittently breathing/apnoea."

It goes on to deal with the treatment that was given after that, but I'd like to focus on this, please: did you or do you recall having any particular contact with [Baby G] on this shift prior to the vomiting incident?

LETBY: No.

MYERS: Right. In particular, had you been asked to look after [Baby G] before the vomiting?

LETBY: No.

MYERS: Do you recall what you had been doing before you were at the nursing station with Ailsa Simpson?

LETBY: No.

MYERS: I'm going to ask if we can take a look in the neonatal review -- and this is something I would ask us to look at, ladies and gentlemen, and you'll need to look at this, please, Ms Letby. It's in the A3 file, the long file. It's on the screen as well, so I'm grateful to Mr Murphy for that.

We're going to go to page 3 of the neonatal review.

Can we enlarge, please, from line 27 down to the event at line 37? In fact, down to 38, please.

I was asking you about any particular involvement that you had had with [Baby G] prior to the incident at line 37.

LETBY: Yes.

MYERS: Again, I'll ask you now: did you have any involvement with [Baby G] before that incident at line 37?

LETBY: No.

MYERS: You're down at lines '33 and 34 as caring for IH.

LETBY: Yes.

MYERS: What time was that at?

LETBY: That's at 2 am.

MYERS: What were you doing with IH at 2 o'clock?

LETBY: I'm carrying out observations and giving a feed.

MYERS: IH is the child you have been designated to look after in nursery 1?

LETBY: Yes.

MYERS: How long might those type of activities take?

LETBY: It's hard to say without looking at the charts, but a matter of minutes.

MYERS: Once you'd finished those, where do you recall being before the events involving the vomit?

LETBY: At the nurses' station with Ailsa Simpson.

MYERS: We can see it has here "[Baby G] event" and underneath that, at line 38, it has a reference to you and nursing notes; can you see that?

LETBY: Yes.

MYERS: And a time of 2.00?

LETBY: Yes.

MYERS: Is that time you put in the nursing notes --

LETBY: Yes.

MYERS: -- as when you took over care?

LETBY: Yes.

MYERS: Is that a precise timing?

LETBY: No.

MYERS: What are you actually doing at 2.00 or thereabouts?

LETBY: Caring for my other designated baby.

MYERS: In fact what was [Nurse E] doing around about 02.00 from what we've seen?

LETBY: She was feeding [Baby G].

MYERS: And we've seen that recorded on the feeding table?

LETBY: Yes.

MYERS: Putting aside timings in the order of events, when were you actually asked to take over the care of [Baby G]?

LETBY: Once [Baby G] had vomited.

MYERS: In fact if we go back, please, to tile 137, that is [Nurse E]'s note -- put this on the screen, if you would, please, Mr Murphy, tile 137. Scroll down to the large entry, top left. Thank you.

If we look at the last couple of lines there. It says:

"L Letby taken over care following vomit/apnoeic episode after 02.00 feed. Her notes to follow."

LETBY: That's right.

MYERS: Is that the right order of events?

LETBY: It is, yes.

MYERS: Had you been involved with [Baby G] in any way before the vomit had happened?

LETBY: No.

MYERS: We can take that down now, Mr Murphy, thank you.

So just following on then from what you're describing --

MR JUSTICE GOSS: Sorry to interrupt, have we finished with the neonatal review files?

MR MYERS: We have.

MR JUSTICE GOSS: I'm thinking if we can put those away.

MR MYERS: We've finished with them for the time being, thank you.

Following on from what you were describing, Ms Letby, you've said that you and Ailsa Simpson heard the monitor, the alarming and vomiting?

LETBY: Yes.

MYERS: Now talk us through, as best you can, what happens after that.

LETBY: On hearing the alarm and the vomiting sound, we both immediately went into [Baby G]. She was only baby in nursery 2 so we knew that the sounds were coming from [Baby G]. We went to her and found her vomiting from her mouth and nose and struggling to breathe.

MYERS: Pause there a moment please. When you say "vomiting from her mouth and nose", do you mean she was in the act of vomiting when you went in?

LETBY: Yes.

MYERS: Was there any sign of vomiting before that point?

LETBY: Yes.

MYERS: You carry on and explain to us what you saw and what happened.

LETBY: When we got to [Baby G] we immediately started to give her Neopuff. We also stripped her off so we could look at her abdomen and it was quite firm and distended and red.

MYERS: Right. What was the lighting like in nursery 1 if you can remember? Sorry, nursery 2. Don't guess if you can't.

LETBY: We could see [Baby G].

MYERS: Was it in darkness?

LETBY: No.

MYERS: Would you ever have a nursery in darkness?

LETBY: No, and certainly not the high dependency room.

MYERS: And that's nursery 2?

LETBY: Yes.

MYERS: Why wouldn't you have something like the high dependency room in darkness?

LETBY: It wouldn't be safe. We need to be able to see the babies at all times -- or at least see their monitors and be able to have access to the babies easily.

MYERS: If you've got high dependency babies, how easy is it to take a view of them if it's in darkness when you walk into the room?

LETBY: It's not.

MYERS: Remaining with [Baby G] and what was taking place, do you recall who used the Neopuff?

LETBY: I think it was myself and Ailsa went to get help.

MYERS: I'd asked you about vomiting: were there any signs of vomiting you could see when you went in there?

LETBY: Yes, when we went to [Baby G] we were both quite shocked because on looking around the scene we could see that there was vomit on the chair that was adjacent to the cot side and also over the floor, which we were both very shocked at.

MYERS: Why were you shocked by that?

LETBY: Neither of us had ever seen a baby vomit to that extent, projectilely (sic) out of the cot. That's not something I've ever seen before.

MYERS: You've described what you and Ailsa Simpson did. Was it just you that remained there, the two of you, or did anyone else come?

LETBY: No, so. Dr Ventress came.

MYERS: How did that come about?

LETBY: A crash call was put out.

MYERS: By crash call, again help us, what do you mean by a crash call?

LETBY: It's the emergency bleep.

MYERS: The emergency bleep?

LETBY: Yes.

MYERS: Where do you go to put the emergency bleep out?

LETBY: It'd be the nearest telephone, so from nursery 2 it would be the nurses' station.

MYERS: And do you know who it was that put that crash call out?

LETBY: I couldn't say, no.

MYERS: Had [Baby G] stopped vomiting by the time you did that?

LETBY: Yes.

MYERS: You said that you were shocked because you'd not seen a vomit like this before; is that right?

LETBY: That's right.

MYERS: Just pausing there a minute, we're going to come, maybe after the break, to 21 September, which is the second event with [Baby G], the event at 10 o'clock in the morning --

LETBY: Yes.

MYERS: -- when you were looking after her. Was the vomiting on the 21st anything like the vomiting we're dealing with on this occasion?

LETBY: No, it was a forceful vomit but not to the extent that we saw on the 7th, no.

MYERS: Do you recall which doctor or doctors did come once you'd put out the crash call?

LETBY: I remember Dr Ventress coming.

MYERS: How was [Baby G] doing by the time Dr Ventress came?

LETBY: She wasn't vomiting at that point and she had stabilised from a respiratory point of view at that point.

MYERS: Did her breathing return to what might be described as a normal stable manner of breathing?

LETBY: No.

MYERS: Did she need any assistance with her breathing going forwards?

LETBY: Yes.

MYERS: What did she need?

LETBY: She needed Neopuffing for apnoeas.

MYERS: Did it become more advanced than that, the breathing support that she was given?

LETBY: Yes, so in the end the decision was made to intubate her.

MYERS: Right. She was intubated that morning?

LETBY: Yes.

MYERS: Again just to help us, because it's a while since we've seen this evidence, I am going to ask to put up some notes by Alison Ventress at tile 80 from the [Baby G] 1 sequence. They are not your notes, I'm not going to be quizzing you about all the content of it, but I think it's helpful if we could, just to keep stock of where we are on this.

This is an entry by Dr Ventress. I'm looking at the one for 04.40 on 7 September. Dr Ventress makes the note that she was called to review [Baby G] urgently at 02.35. The first part of this is reporting what she received, what information she had. I'm going to ask in fact if we can go to the following page at tile 107.

If we scroll down that page, we can see what you've described, that [Baby G]...

"IPPV given and gradual improvement again."

Can you see that, Ms Letby?

LETBY: Yes.

MYERS: So in relation to what you were describing, what was happening when she was given IPPV, what was happening then?

LETBY: So she was responding to that -- her saturations were improving.

MYERS: Two lines below that:

"Decision to intubate due to two profound desaturations."

LETBY: Yes.

MYERS: Do you remember the detail of this?

LETBY: No.

MYERS: Do you remember that he did continue to have desaturations?

LETBY: Yes, I remember she continued to have desaturations, not any details of them, though.

MYERS: And you recall that she had to be intubated?

LETBY: Yes, and I remember Dr Ventress was not on the unit at that point.

MYERS: Do you recall whether, having been intubated, that solved the problem of her oxygen saturation or whether the problems continued?

LETBY: They continued.

MYERS: Do you know why the problems continued?

LETBY: No.

MYERS: Did you play a part in the continuing support that was being given to [Baby G]?

LETBY: Yes.

MYERS: Did you carry on with that throughout the course of the shift?

LETBY: Yes, I did.

MYERS: Do you recall whether there was any resolution to the problems that [Baby G] had been experiencing whilst being ventilated, whether it was sorted out?

LETBY: I know there was a point when [Baby G] was re-intubated and the ventilator circuit was changed and she responded.

After that, she did improve but she obviously still remained unwell.

MYERS: And that was during the later part of this morning?

LETBY: Yes.

MYERS: Of the shift?

LETBY: Yes.

MR MYERS: Thank you.

My Lord, I'm going to move to count 8, which is 21 September. I can start that now or we can take the break, whatever is more convenient for the court.

MR JUSTICE GOSS: Entirely up to you.

MR MYERS: Perhaps if we were to conclude now, that's a natural place to stop.

MR JUSTICE GOSS: Certainly. We'll have the 15-minute break then, please. So ready to resume at 11.50, please, members of the jury.

(11.35 am)

(A short break)

(11.51 am)

MR MYERS: Count 8, Ms Letby. We're going to move to count 8 now, which is 21 September. Counts 8 and 9 are both on 21 September: count 8, some time after 10 o'clock in the morning, and count 9, a little bit after 3.15 in the afternoon.

Just to remind ourselves, in between what we're dealing with before the break and what we are turning to now, [Baby G] had been transferred to Arrowe Park Hospital and she was there between 8 September and 16 September.

So we have just dealt with 7 September. On the 8th [Baby G] went to Arrowe Park and she returned to the Countess of Chester on the 16th and we move forwards now to the 21st.

And let's have a look at the layout, if we could, please, Mr Murphy: tile 45 from sequence 2 for [Baby G]. If we just go behind that. Thank you.

Can we have a look, first of all, at who was on this shift? You're there, of course, Ms Letby. This is 7.30 in the morning this starts. The shift leader is Caroline Bennion. We can see it's referring to [Nurse B], Shelley Tomlins and Caroline Oakley and also Janet Cox is the nursery nurse.

Can you see that, Ms Letby?

LETBY: Yes.

MYERS: Where it says:

"Management: Eirian Powell, Yvonne Griffiths and Yvonne Farmer."

I know we had some evidence from Eirian Powell as a witness on this, but were Eirian Powell, Yvonne Griffiths and Yvonne Farmer on the unit that day?

LETBY: Yes.

MYERS: Were they caring for babies?

LETBY: No, so they're in a non-clinical role, they have an office on the unit that -- they're doing office-based work but they are on the unit.

MYERS: Right. Are they -- we've seen how someone who is supernumerary, usually a shift leader, may be called in to help with a baby.

LETBY: Yes.

MYERS: If you've got management staff, like these nurses, present, are they called in to help with babies or are they there for a different reason?

LETBY: No, they're there to do office-based work usually.

MYERS: Right. They're not getting involved in the care?

LETBY: No.

MYERS: I only ask that so no one is under any misapprehension that although they're in this box, they're looking after babies?

LETBY: No, they'd be present on the unit but they would not be involved with clinical care.

MYERS: Thank you. Let's scroll down then with Mr Murphy's assistance to see who is where, with a particular interest with where you were, Ms Letby. Who were you caring for on this day?

LETBY: So I had three children in nursery 4 and also a baby that was rooming-in with parents.

MYERS: Let's pause a minute. In nursery 4, top left-hand corner, we've got [Baby G] and two other babies?

LETBY: Yes.

MYERS: You were looking after them?

LETBY: Yes.

MYERS: Down in the box, lower right, we can see it says OR was rooming-in with parents.

LETBY: Yes.

MYERS: Can you explain to us what that means?

LETBY: Yes. When a baby is preparing to go home, as part of the discharge planning we offer the parents the chance to stay in

the parent accommodation overnight to care for their baby with staff on site for support if needed.

So yeah, there's family accommodation and that's where the parents are looking after their baby.

MYERS: And is that accommodation in the neonatal unit?

LETBY: It's just off the unit.

MYERS: Right. Because we know there's secure doors around the neonatal unit.

LETBY: Yes -- no, it's not within a secure door, it's just --

MYERS: Right. And would the baby be with them in the room that they are in?

LETBY: Yes.

MYERS: So if you're looking after three babies in nursery 4, what are you having to do so far as baby OR is concerned who's rooming-in with the parents?

LETBY: So that would be going into the parent accommodation periodically to check that the parents and the baby are okay and to be on hand if the parents have any questions.

MYERS: Right. I'm going to go next to your note for this, Ms Letby, and then we'll have a look at the particular events. Can we put up tile 48, please. The top left hand -- 12.47:

"Written for care given from 08.00. Emergency equipment checked. Fluids calculated. [Baby G] nursed in a cot. [Baby G] appears pale. Temperature 36.4. Hat in situ and well wrapped. NG tube feed. EBM given [at] 09.00 as [Baby G] asleep and due immunisations. At 10.15, x2 large projectile milky vomits, brief self-resolving apnoea and desaturation to 35% with colour loss. NG tube aspirated. 30ml undigested milk discarded. Abdomen distended, soft. Doctors asked to review. Temperature remains low. Tachycardic..."

Does that mean above 180 beats per minute?

LETBY: Yes.

MYERS: Tachycardic meaning what?

LETBY: Fast heart rate.

MYERS: "... since vomit. Mum states that [Baby G] does not appear as well as she did yesterday."

Then it's got other checks that are conducted and actions taken and it says:

"Care handed over to Staff [Nurse B] at approximately 11.30."

Do you remember the incident that this describes?

LETBY: Yes.

MYERS: Did you give a feed at 09.00 as it says here?

LETBY: Yes.

MYERS: I'm going to ask to put up tile 47, which deals with that feed and then I've got some questions about that, please.

Tile 47 in [Baby G] sequence 2. Which of these is the feed that you are referring to, Ms Letby?

LETBY: The 09.15.

MYERS: What does that tell us you did and what happened with regard to this feed? Take your time to talk us through that if you would, please.

LETBY: I've given a 40ml feed of expressed breast milk via the NG tube at 09.15. The comments that come after that follow the feed so that's after the feed.

MYERS: So 40ml expressed breast milk. Where it says "30ml x2 milk projectile vomit" and a D, can you see that?

LETBY: Yes.

MYERS: Can you tell us what all of that relates to?

LETBY: That is after the feed. So after the feed there were two projectile milky vomits, I've aspirated the NG tube and got 30ml, which I have then discarded and at that time she's also had a large bowel motion as well.

MYERS: Right.

LETBY: This is post-feed.

MYERS: Post-feed. So is this referring to what your note refers to when it describes the vomit --

LETBY: Yes.

MYERS: -- round about 10.15?

LETBY: Yes.

MYERS: What does the 9.2 stand for?

LETBY: So [Baby G] had a blood gas taken after her vomit and that was the blood sugar reading.

MYERS: Is that high?

LETBY: Yes.

MYERS: I know you mentioned this, "BO +++"; what does that relate to?

LETBY: That's a large bowel movement and it was a loose watery green stool which is abnormal.

MYERS: And again when in the sequence of events did that pass?

LETBY: Again this is post-feed, so this is once the vomit had happened.

MYERS: And "Reviewed by doctors", is that what it says there?

LETBY: Yes, she was reviewed on reflection of the vomit and the loose stool.

MYERS: Would you have tested the pH before you did this feed?

LETBY: Yes.

MYERS: Would you have aspirated the stomach contents before you did this feed?

LETBY: No.

MYERS: Is there any reason or any guidance as to when a baby's stomach contents are automatically aspirated prior to a feed and when they're not? Is there any kind of rule for that?

LETBY: So usually, with a baby that's -- I mean if we look, [Baby G] here is 40 weeks' gestation, so she's now a term baby. We wouldn't therefore need to fully aspirate all of her milk out of her tummy at every feed.

MYERS: Pause there a minute. You say she's 40 weeks gestated.

Are you looking down the left-hand --

LETBY: Yes. It says "corrected gestation" on the left-hand side.

MYERS: Corrected gestation. So we know --

LETBY: So we need to start, at this point, treating [Baby G] as we had a term baby and with a term baby we would not fully aspirate their milk at every feed as it disrupts the digestion process.

MYERS: Would you know in advance of a feed if there's milk already in the stomach or, if there is, how much?

LETBY: No. You would only aspirate enough to check the tube placement.

MYERS: Do you recall how [Baby G] seemed to be after you'd given the feed at 9.15?

LETBY: I don't recall from memory.

MYERS: The notes that you've written in, you have told us about where we can see "30ml x2 projectile vomit", those notes, you've described those are things you wrote after the feed?

LETBY: Yes.

MYERS: So they're written on the chart after that took place?

LETBY: Yes.

MYERS: Was this vomit anything like the vomit that had happened on 7 September that morning?

LETBY: No.

MYERS: And when you say no, what was the difference?

LETBY: It was not anywhere near the volume or the projection.

It was a projectile vomit, but it did not leave the cot space on this occasion.

MYERS: How many millilitres had you fed [Baby G] before the vomit?

LETBY: 40.

MYERS: And how many did you get out?

LETBY: 30.

MYERS: What did you do with those?

LETBY: That 30ml was discarded, so it's put in the bin.

MYERS: Do you know what happens to the 10ml, the difference between the 40 and the 30?

LETBY: I would assume that's the vomit, she's lost 10ml.

MYERS: Was there anybody else in the nursery at the time this happened?

LETBY: I can't be sure from memory if mum was present at this time or not.

MYERS: Had mum been present at various points during [Baby G]'s care?

LETBY: Yes, very much so.

MYERS: And on this day?

LETBY: Yes.

MYERS: Let's just look at tile 48 if we could, please, again which is the notes you made. Just go back to that.

Could we enlarge the top left, please, and scroll down to the family record. Pausing there, this is a note -- who made this note at 12.52?

LETBY: I did.

MYERS: What does it say? Read it for us, please.

LETBY: "Mummy telephoned this morning. Arrived shortly after 10.00. Fully up to date on [Baby G]'s condition this morning ing and current management. Aware that she will not have her immunisations today."

MYERS: Do you remember what precise moments [Baby G]'s mother was in the room and when she wasn't in the room?

LETBY: No.

MYERS: Was she in the unit on or after 10 o'clock in the morning?

LETBY: Yes.

MYERS: Do you recall whether she was there at the time that [Baby G] vomited or do you not recall specifically?

LETBY: I couldn't say specifically.

MYERS: Were you there when [Baby G] vomited?

LETBY: No, I was -- I went to her (inaudible) vomiting.

MYERS: What alerted you to the vomiting?

LETBY: Her monitor.

MYERS: Was there anybody with her when she vomited as far as you can recall?

LETBY: No.

MYERS: Do you know where you were when you were alerted to it?

LETBY: I believe I was in nursery 4, but I wasn't at [Baby G]'s cot side --

MYERS: So you're in the same room?

LETBY: Yes.

MYERS: But not dealing with [Baby G] at that point.

LETBY: Yes.

MYERS: And you had two other babies in there?

LETBY: Yes.

MYERS: What did do you once the vomit had taken place?

LETBY: [Baby G] stabilised after that. The doctors were on the unit, starting to do the ward round. I went down to the nurses' station and just asked if possibly [Baby G] could be seen sooner because usually nursery 4 would be the last nursery to be seen, so I just asked if they would be able to see [Baby G] prior to last.

MYERS: Was there any need to put out a crash call?

LETBY: No.

MYERS: Any need to shout for help?

LETBY: No.

MYERS: Why not? Why was it not necessary to do those things on this occasion?

LETBY: Because [Baby G] hadn't needed any assistance, she'd self-corrected with her apnoea. After the vomit she recovered and she seemed well. There was no intervention required.

MYERS: Was there any what might be called large scale medical response required for what happened?

LETBY: No.

MYERS: Any extraordinary changes in colour or anything like that?

LETBY: No.

MYERS: Why was care transferred to [Nurse B] after this incident?

LETBY: So upon review from the doctors, she required a high level of care at that point and I had three other babies, so it was decided that [Nurse B] would take over the care for [Baby G].

MYERS: That was what took place in the morning. We know there's another incident involving [Baby G] later that day in the afternoon round about 15.27; that's count 9 on this indictment. So let's move forwards to 15.27 or thereabouts.

By the time we get to the afternoon, was [Nurse B] the designated nurse?

LETBY: Yes.

MYERS: Was [Baby G] still in nursery 4 though?

LETBY: Yes.

MYERS: And did you still have other babies to look after in nursery 4?

LETBY: Yes.

MYERS: And as well as you can remember, were you also looking after that baby that was rooming with the parents?

LETBY: Yes.

MYERS: You've described that parents are able to come on to the unit at any time apart from handover; is that right?

LETBY: That's right, yes.

MYERS: So would there routinely be parents on the unit at about 3 o'clock in the afternoon?

LETBY: Yes, so parents are very much present in the daytime, yes.

MYERS: Do you know when they're going to arrive?

LETBY: No.

MYERS: Do you recall whether there were parents around as we're getting to 3 o'clock in the afternoon with [Baby G]?

LETBY: Yes, there were, yes.

MYERS: Is there any reason why you remember that?

LETBY: Because I know when I found [Baby G] that I was conscious that there were other parents in the room at the time.

MYERS: Right. This is when you're talking about the incident at 3 o'clock that afternoon and other parents were there?

LETBY: Yes.

MYERS: Do you recall there were screens that were put around [Baby G]?

LETBY: There were, yes.

MYERS: Why are screens put around a baby on the unit?

LETBY: So screens are put around a baby to give them privacy from other parents in the room if they're having a procedure undertaken. In this case, [Baby G] was having cannulation, so it's normal practice for a screen to be put round to give that baby privacy.

MYERS: All right, thank you. We'll have a look at what [Nurse B] put in her note and then we'll go through your recollection of events with [Baby G] that afternoon. So can we go to tile 72, please. In fact, we're there. Can we scroll down to the bottom of the page or the lower left-hand side.

21 September 2015. This is a note at 17.20 by [Nurse B]:

"Care taken over at approximately 11.30 due to staff workload."

Is that what you described earlier about [Baby G] being reallocated?

LETBY: Yes.

MYERS: "[Baby G] pale/asleep in cot. Masimo monitor in situ. Self-ventilations in air. Initially tachycardic at 175 following vomiting episode documented by Nurse L Letby in previous notes. Cannula inserted first attempt by Reg Chang but would not advance past junction but able to give IV ABs via it."

What are IV ABs; are you able to help us?

LETBY: IV antibiotics.

MYERS: That's intravenous antibiotics?

LETBY: Yes.

MYERS: "Bloods obtained for culture. CPR 18, Hb 95."

Is that haemoglobin, Hb?

LETBY: Yes.

MYERS: "Lumbar puncture required due to symptoms but not yet performed. Numerous failed attempts then at cannulation. Finally inserted by Dr Gibbs. Without fluid for 6 hours as NBM."

Is that nil by mouth?

LETBY: Yes.

MYERS: "... as [nil by mouth]. Blood sugars were stable throughout. All oral medications omitted as per Reg Chang. Further significant apnoea/brady/desat following cannulation, requiring Neopuff and 100% oxygen. Help summoned and transferred to nursery 1 and placed in incubator for close observation."

It then describes the steps taken after that.

Before we look at this in a little more detail, where it says, "Further significant apnoea/brady/desat", who was it that discovered [Baby G] was having apnoea/brady/desat?

LETBY: It was myself.

MYERS: What did you do when you discovered that?

LETBY: I called for help.

MYERS: Do you recall what it was that had been going on with [Baby G] prior to that event taking place?

LETBY: Yes, so I recall that [Baby G] had been behind the screen for quite a prolonged period of time while doctors were trying to cannulate her.

MYERS: Were you involved in that process at all?

LETBY: No.

MYERS: Were you chatting with those doctors at all?

LETBY: No.

MYERS: What were you occupied by?

LETBY: The other babies that I was caring for.

MYERS: Two in that nursery?

LETBY: Yes.

MYERS: And one outside, in an outside room?

LETBY: Yes.

MYERS: Were you aware that it took a little while or some effort for [Baby G] to be cannulated?

LETBY: Yes.

MYERS: Can we put up tile 71, please, just to remind ourselves of this? 21 September. Type of line, left foot.

Inserted by Dr Gibbs, we see, on the 7th attempt by multiple staff. Were you present when these attempts were ongoing as far as you know?

LETBY: I may have been in the nursery but I was not present with [Baby G].

MYERS: Not with [Baby G]?

LETBY: No.

MYERS: Was the screen up while this was taking place?

LETBY: It was, yes.

MYERS: Thank you, Mr Murphy.

What led to you discovering what had happened with [Baby G]?

LETBY: It's common practice when doctors are doing a procedure and there's a screen up in the nursery that periodically staff would look behind the screen just to check that everything's okay. I can't remember exactly why I went but I happened to catch sight behind the screen and saw that [Baby G] was on her own, behind the screen.

MYERS: Pause there a moment. When you saw that she was on her own behind a screen, could you see what condition she was in?

LETBY: She was dusky and blue and not breathing.

MYERS: Right. What about the monitor?

LETBY: The monitor wasn't on.

MYERS: What did you do when you discovered her in that condition?

LETBY: At this point [Baby G] was on the procedure trolley, so I immediately picked her up and put her into the cot and started Neopuffing her.

MYERS: Again, just to help us, can you explain to us what you mean by the procedure trolley and then the cot?

LETBY: So the procedure trolley is a trolley -- it's like a flat bed with no sides on that is used for doing procedures on babies so that we've got access around the whole of the baby. Babies are never left unattended on that because they do not have sides so potentially the baby could fall. At this point [Baby G] was on the trolley and the Neopuff wouldn't have stretched that far.

MYERS: So when you checked on her, she's on the procedure trolley?

LETBY: She was, yes.

MYERS: Should she have been left on the procedure trolley?

LETBY: No, that's not standard practice at all.

MYERS: Was anyone with her?

LETBY: No.

MYERS: Is there any circumstances where the doctor should have left her or the nurse that was with the doctor should have left her like that?

LETBY: No.

MYERS: You mentioned that you'd moved her, I think. Where did you move her to?

LETBY: I picked her up and put her straight back into her cot and was then able to access the Neopuff and started Neopuffing her and called for help.

MYERS: You called for help?

LETBY: Yes.

MYERS: And did anyone come?

LETBY: Yes, [Nurse B].

MYERS: Had you had any involvement with [Baby G] after handing her over to [Nurse B] before this point?

LETBY: No.

MYERS: Do you know why the monitor was off?

LETBY: No.

MYERS: Did [Nurse B] come over?

LETBY: She did.

MYERS: Just describe for us what happened following on from that.

LETBY: [Nurse B] appeared at the nursery and I said I needed some help. She froze, actually went out of the nursery and got somebody else to come and assist me and at that point Caroline Bennion came in.

MYERS: Did you appreciate what had happened with the doctors?

LETBY: What do you mean, sorry?

MYERS: Well, did you have anything to say about what it was that you'd discovered there?

LETBY: Yes, I was very concerned by it.

MYERS: Could you explain to us, so we understand, what you were concerned about and what happened because you were concerned?

LETBY: There were three issues, really. It was -- we would never leave a baby unattended on a procedure trolley.

We would also never leave a baby unattended behind a screen. And we'd never leave a baby without a monitor on. Those three things had happened so I was quite concerned that [Baby G] had been left in that situation.

MYERS: Did you raise those concerns with anybody?

LETBY: With [Nurse B], yes.

MYERS: Was any action taken?

LETBY: We had a conversation about it and I was quite keen to put a Datix form in, which is the clinical incident form. [Nurse B] was reluctant for this because it was a consultant, it was Dr Gibbs, and she assured me that she would deal with the situation herself and would talk to the ward manager, Eirian Powell, about it.

MYERS: Pause there a moment. Were you aware of Dr Gibbs and Dr Harkness returning later that afternoon to apologise to [Nurse B]?

LETBY: No.

MYERS: You've heard the evidence from [Nurse B] about that. And the questions --

LETBY: I can't recall that --

MYERS: You don't recall that?

LETBY: -- if that happened or not, no.

MYERS: She has described that happening.

LETBY: Yes.

MYERS: Did you take the matter any further?

LETBY: No.

MYERS: Why didn't you take it in further?

LETBY: [Nurse B] and I had discussed it, she was [Baby G]'s designated nurse and she is a senior member of staff.

I took assurances that she would deal with it as she had said.

MYERS: All right. That incident is on the afternoon of 21 September. In fact, [Baby G] remained on the unit for over a month after that. Again, I'm not going to go through all of the material now, we'll return to it, ladies and gentlemen, at a later stage.

You may recall when Dr Bohin gave evidence we went over a series of vomits a related incidents involving [Baby G] from 8 October through to 23 October.

LETBY: Yes.

MYERS: Did you have other occasions of caring for [Baby G] after this particular day?

LETBY: Yes.

MYERS: All right, thank you. We'll look at it another way of assisting with the jury with the dates that were put to Dr Bohin.

Having looked at what happened with [Baby G], I would like to turn next to [Baby H]. If we just pause to give me one moment to catch up with myself.

(Pause)

We'll deal with a little summary to take us back to the events of [Baby H] who is the baby, ladies and gentlemen, with the three chest drains. You may remember that. That's where we come to next.

Ms Letby, we are dealing with counts 10 and 11 now on this indictment.

LETBY: Yes.

MYERS: Just to remind ourselves about the introductory details with [Baby H], could we put up tile 2, please? Just behind that. Thank you very much, Mr Murphy.

So [Baby H], born on 22 September 2015 at 18.22, a 34-week and 4-day gestation. 2.33 kilograms.

Born, in fact, after an emergency caesarean section.

The events that we're looking at were on the 26th and then 27 September. I'm going to come to those in a moment, but other

aspects of the chronology, by no means all of it, but other aspects that aren't on the sequence of events, so I'm going to remind us of it now.

[Baby H] was born on 22 September 2015 at 18.22. She received surfactant -- or rather didn't receive surfactant until 11.30 on 24 September 2015, which is 41 hours after her birth. So that's when she received surfactant, 11.30 on 24 September.

10 am on 24 September is the time of the first chest drain, inserted by Dr Harkness. I'm giving you the detail of that, Ms Letby and members of the jury, because that isn't on the sequence of events, but you'll recall there were three drains in total. Drain 1 was on 24 September in the morning. Drain 2 was on 25 September at 3.00 am and that was inserted by Dr Jayaram. Drain 3, almost 24 hours later, is 26 September 2015 at 2.15 am by Dr Gibbs.

For everyone's ease of reference, the second drain, at 3 o'clock in the morning of the 25th, the details relating to that are on tile 76. That third drain with Dr Gibbs at 2.15 on the 26th is on tile 243. That's his note relating to that. That's really just for everyone's ease of reference.

Then we come to count 10. That's at 3.22 on the morning of the 26th, a little more than an hour after the third chest drain was inserted. At 3.22 that morning, there was a profound desaturation to 30%.

We'll look at some of the details of that shortly.

Count 11, the next event, is the following morning, early morning, 27 September at about 00.55, when there was a desaturation to the 40s. So that's just to give us a basic chronology of what took place and when.

Born on 22 September, the three drains in over the 24th to the 26th, and then the two events: 3.22 on the morning of the 26th is count 10, 00.55 on the 27th is count 11.

Outside what is on the medical notes for [Baby H], Ms Letby, do you have any significant degree of recollection of events concerning her?

LETBY: I remember [Baby H] and I remember her because of her care needs, but in terms of specific details, no, not without referring to the notes.

MYERS: Right. Do you recall, in general terms, how busy the unit was over this period? We're talking in particular with [Baby H] during September 2015.

LETBY: Yes, the unit was busy at that time.

MYERS: I'm going to ask if we could put up tile 27, which is a message. Thank you, Mr Murphy. 9.15 on the morning of 24 September, the morning, as it happens, that the first chest drain was put in.

It's from you to [Nurse E]. The message says:

"It's completely unsafe [and an emoji]. Yeah, I told Yvonne F that and she's going to look into it. I still have to do next Wednesday day as can't cover it, but getting paid as overtime for last night x"

What's the reference about "It's completely unsafe" to?

LETBY: The staffing levels on the unit.

MYERS: Why would you be telling Yvonne Farmer about that?

LETBY: Yvonne Farmer is part of the management team.

MYERS: Is she one of the names that we saw in that management meeting hen we last looked at the --

LETBY: She is, yes.

MYERS: -- [Baby G] event on 21 September?

LETBY: Yes.

MYERS: So she's involved in management?

LETBY: Yes.

MYERS: Can we go next, please, to a message -- sorry, to tile 33, which is a message from you to Sophie Ellis later that day. 14.07 in the afternoon on the 24th:

"Oh Soph, it was pretty bad: 18 babies, intubating on handover and a baby with a sugar of 0.1! On again tonight tomorrow and sat. No had chance to see Corrie. Was it good?"

Is that Coronation Street, Corrie?

LETBY: Yes.

MYERS: Putting that to one side for a moment if we can. When it says, "Oh Soph, it was pretty bad", what's that relating to?

LETBY: I'm referring to my shift.

MYERS: What's bad? What was bad? Just tell us.

LETBY: I'm stating there we've got 18 babies, which is over capacity, the unit catered for 16 and we have now got 18, and there was a lot going on. Obviously we were intubating on handover and there was another very poorly baby with a low blood sugar.

MYERS: Had the unit been as busy as this throughout all the years you'd worked there?

LETBY: No.

MYERS: Ms Letby, you'll have to raise your voice a little bit.

LETBY: Okay, sorry.

MYERS: What I'd asked was whether the unit had always been as busy as this on all the years you'd worked there?

LETBY: No, it was increasingly busy at this period.

MYERS: Had you got any experience of babies with chest drains before dealing with [Baby H]?

LETBY: I'd never seen a chest drain at the Countess of Chester, but I had come across them at Liverpool Women's during my placements there.

MYERS: How common was it to have -- could you form any idea of how common it was to have chest drains with babies on the Countess of Chester?

LETBY: Well, I can't say that I'd ever seen one in my time there up until [Baby H].

MYERS: Had you ever seen a baby with three chest drains before?

LETBY: No, never.

MYERS: Not even at a tertiary unit?

LETBY: No, the most I'd seen is two.

MYERS: Were you able to form an opinion as to how familiar your colleagues were in looking after a baby with chest drains?

LETBY: None of us were overly familiar because it is something that happened quite rarely on the unit.

MYERS: And so far as the doctors were concerned, could you form any assessment as to how familiar they were with dealing with chest drains?

LETBY: They weren't familiar either.

MYERS: Did you see anything which tended to support that view as to why they weren't familiar?

LETBY: Quite often they would have to look things up, how to do things, or there would be discussions on how they managed drains. Also it was more than one drain in use and there was a discussion about how each drain is cared for differently.

MYERS: The first event we come to with this indictment is in the early hours of the morning of 26 September, by which point [Baby H] had three drains.

LETBY: Yes.

MYERS: And she'd had them for just under an hour -- or just over an hour by that point. Just over an hour. Were you aware of her on the unit before that point?

LETBY: Before?

MYERS: Were you aware of [Baby H]'s course on the unit generally when she was being cared for?

LETBY: I'm not sure.

MYERS: All right. How are the chest drains meant to be kept in place?

LETBY: From my experience at Liverpool Women's Hospital, the drains were always sutured in place.

MYERS: What do you mean by that?

LETBY: So stitched by the doctors, stitched into the skin.

MYERS: Do you know why the doctors had done that from your experience elsewhere?

LETBY: That's the safest way to secure the drains, to make sure they don't move.

MYERS: Did you keep chest drains on the unit?

LETBY: Yes, but very few.

MYERS: Do you know where the chest drains were taken from that were used for [Baby H]?

LETBY: Yes, so we had one drain on the unit in the equipment room, which was used. We then used the emergency drain that was on the resuscitation trolley, meaning that we then had none left on the unit, so the third one was then obtained from the children's ward.

MYERS: Right.

LETBY: And we had to get those replaced then by Arrowe Park.

They couriered out some drains to replace those.

MYERS: When you cast your mind back to the events, particularly the events over these two nights or early mornings, with the collapses, without the notes are you able to pull out specific details and analyse it all and recall exactly what happened and when?

LETBY: No.

MYERS: And do you recall in particular, without notes, what it was you did after the collapses took place?

LETBY: No.

MYERS: Or were doing before the collapses took place?

LETBY: No.

MYERS: I'm going to ask if we can just put up, by way of an example, page 31. The first page will be sufficient, Mr Murphy. Page 31 from the prosecution images. Just look at the date on this to start with. We can see at the top, this is a handover sheet, Ms Letby, for 23 September 2015. Can you see that?

LETBY: Yes.

MYERS: This is when [Baby H] had -- she'd been born on the 22nd and this is one of the handover sheets recovered your home when the police went in 2018 --

LETBY: Yes.

MYERS: -- from the Ibiza bag. (Pause). Sorry, the Morrisons bag, from the Morrisons bag.

[Baby G]'s name appears on the handover, so the handover is -- four more handover sheets with her name on like this?

LETBY: Yes.

MYERS: I'm just taking this one at this point. I know I asked you about this when you began your evidence, but can you help us: why is it that you've got a handover sheet for [Baby G] in that bag when the police come to your house several years later? Sorry, [Baby H], my mistake, I apologise. And [Baby G].

LETBY: So it has just come back with me from work inadvertently and got left at home.

MYERS: I'm looking at the names. We can see there's three of the names of babies from this case there: [Baby H], [Baby G] and [Baby I]. Can you see?

LETBY: Yes.

MYERS: It's not just this one, there are another four with [Baby H]'s name on --

LETBY: Yes.

MYERS: -- and some of the other names on over this period.

LETBY: Yes.

MYERS: So again, I'm going to ask you: how is it that they find their way from the unit to you back home?

LETBY: They have not been taken out of my pocket at the end of the shift and have come home with me and I have not disposed of them.

MYERS: Did you mean to take them home?

LETBY: No.

MYERS: We've heard that there were, I will just remind us all, 257 handover sheets in total.

LETBY: Yes.

MYERS: 31 relating or including the names of children on this indictment.

LETBY: Yes.

MYERS: Did you never think to yourself: well, I've got quite a lot of these handover sheets, I think it's time to get rid of all this lot?

LETBY: No, I did not recall having that many. I did not know there was that amount in my house.

MYERS: Did you keep track of what you had?

LETBY: No.

MYERS: Let's have a look at the event on 26 September in the early morning, which is count 10, so if we take that down and put up tile 177. This is that night shift.

Dr Gibbs is the paediatrician of the week and he was on call and we know he fitted the chest drain or inserted the chest drain at 2.15. In terms of nursing staff, [Nurse A] is the shift leader, you're the designated nurse, Yvonne Griffiths is on duty, and Nurses Thomas and Marshall also present.

What I'll do, given what you said about recollection, Ms Letby, is go to the note you made, we'll have a look at that, and then I'll have some questions for you about it.

LETBY: Okay.

MYERS: I'm going to ask next if we can come to tile 261. It's the start of the note. Tile 261 for [Baby H]. The note starts -- if we scroll up a little bit, Mr Murphy, we're not going to read all of it, but obviously it's there for any other details to be extracted if there are other questions to ask about it.

26 September, 04.14 that morning.

LETBY: Yes.

MYERS: This is made after the event?

LETBY: Yes.

MYERS: By you?

LETBY: Yes.

MYERS: "Written for care given from 20.00 to present."

So from that shift. It sets out the various checks that are conducted at the start of the shift. The next paragraph down begins:

"x2 chest drains in situ at start of shift. Intermittently swinging. Serous fluid ++ accumulating."

I'll come back to some of the detail in a moment:

"23.30. Bradycardia and desaturation requiring Neopuff in 100%..."

100% what, by the way?

LETBY: Oxygen.

MYERS: "... oxygen to recover. 10ml..."

Should that be "air"?

LETBY: Yes.

MYERS: "... aspirated from chest drain by Reg Ventress.

Following poor blood gas and 100% oxygen requirement, Consultant Gibbs attended the unit and inserted a third chest drain. All three drains swinging and serous fluid present. Dressing in situ. X-rays taken as documented."

Then sets out some medication. IV fluids below that:

"Babiven via long line. Lipid unable to run as line occluding."

Occluding?

LETBY: It means the line isn't running fully so it won't infuse the lipid, which is a thicker liquid.

MYERS: All right:

"Doctors aware. Medications given as prescribed. Blood transfusion completed at 02.00 via peripheral line."

I'll come back to that:

"Observations within normal range. Sodium chloride bolus at 02.50. Mean blood pressure 33. [Baby H] desaturating ++ on handling. Minimal handling observed when possible. At 03.22, profound desaturation and colour loss to 30%. Good chest movement and air entry. Colour change on CO2 detector. Neopuff commenced in 100% oxygen and help requested. Serous fluid ++

from all 3 drains. Became bradycardic. Doctors crash called and resus commenced as documented. Consultant Gibbs in attendance. X3 doses adrenaline and x1 dose atropine given via peripheral line. Chest compressions stopped at 03.46."

After which, relatively speaking, [Baby H]'s condition was stabilised. It sets out other steps there, but it's the event at 03.22 in particular that I'm focusing on with you, Ms Letby.

Outside what is written in this note, again I'm going to ask, do you have particular recollections of what happened with [Baby H] on this early morning?

LETBY: Not with any specific detail, no.

MYERS: Just in terms of the detail, you refer in two different places to chest drains and about them swinging; you describe, when there were two, them intermittently swinging and when there were three fitted, the third one had been fitted, you talk about "chest drains swinging".

What are you describing when you say that?

LETBY: Swinging is the nursing term that we use to say that the drains are working and by swinging it means that we can see serous fluid, so just bodily fluid, moving in and out of the drain, which shows the drain is working.

MYERS: And when you say in and out of the drain, which bit -- what are you looking at?

LETBY: There's a clear tubing that comes out of the drain that attaches to either a pot or a valve. And swinging means that the fluid is moving back and forth, which means air is coming in and out of the drain as needed.

MYERS: So after 2.15, would there have been three tubes coming out of [Baby H]'s chest --

LETBY: Yes.

MYERS: -- three drains?

LETBY: Three separate drains with three separate tubings, yes, and each of those were swinging with serous fluid.

MYERS: Serous fluid, is that anything in particular?

LETBY: Just like a straw-coloured fluid that's just natural fluid in the body.

MYERS: Where does that fluid go as it comes through the drain?

LETBY: Depending on which drain you've got, it either drains into a pot or it just collects at the end of a valve.

MYERS: Were you the designated nurse for [Baby H] during this shift?

LETBY: Yes.

MYERS: Just put up tile 177 for a moment, please. [Baby H] is in nursery 1. Was she the only baby in nursery 1 from your recollection?

LETBY: Yes.

MYERS: Did anybody give you assistance in looking after [Baby H] as she was in nursery 1?

LETBY: Yes, [Nurse A] was the shift leader and she was supernumerary, so that was the idea, that she was there to support me with [Baby H].

MYERS: Why was it necessary for her to be there to support you with [Baby H]?

LETBY: [Baby H] was requiring a very high level of care at that point, quite often needing two nurses to be with her.

MYERS: So what did that mean if you're saying [Nurse A] was there to give you support, what was it she was doing?

LETBY: Assisting me with any of the drugs that were needed and also to assist with maintaining the chest drains.

MYERS: Did you have to call for her from time to time?

LETBY: Yes.

MYERS: Did she ever attend when you hadn't called for her?

LETBY: [Nurse A] was in and around quite a lot of the time.

MYERS: Is she the only nurse who would come in nursery 1, in and around nursery 1?

LETBY: No, nursery 1 is predominantly where all of the drugs and equipment are kept, so nurses and nursery nurses are in and out of that nursery a lot of the time as well as doctors.

MYERS: Thank you. If we look back at your note, which is at tile 261, there's another detail I would like to ask you about, which is right in the middle of the entry, just before we come to the desaturation at 3.22. A little bit up from there. Thank you.

Can you see it says:

"Blood transfusion completed at 02.00 via peripheral line"?

LETBY: Yes.

MYERS: As this case has gone along, you've had the chance, as we have, to see the papers which are going in front of the jury and we are all looking at in court?

LETBY: Yes.

MYERS: Is that accurate where it says:

"Blood transfusion completed at 2.00"?

LETBY: No, that's an error.

MYERS: What time was that blood transfusion dealt with?

LETBY: 3 o'clock.

MYERS: 3 o'clock. Can we put up tile 239, please. Looking at this, what is this document, please, Ms Letby?

LETBY: So this is a specific chart that's used for the prescription of a blood product.

MYERS: Is that what we call a transfusion?

LETBY: Yes.

MYERS: What does this tell us happened?

LETBY: So this is -- shows that packed red blood cells, so just a blood transfusion, was given at 47ml over 4 hours and it was given from 23.05 on the 25th, finishing at 03.05 on the 26th.

MYERS: So judging by this, what time should have been put in your note for the blood transfusion having been given?

LETBY: 03.00.

MYERS: Who has written 03.05 in here, do you know?

LETBY: I'm not sure.

MYERS: Who signed for that?

LETBY: It's myself and [Nurse A].

MYERS: Who's able to look at this document in the records?

LETBY: Anyone. That is kept within the baby's documents, so it's accessible for anyone that needs to see it.

MYERS: Could we look also at tile 242, which is the observation chart, just dealing with the same thing. Tile 242, please, Mr Murphy.

I'd be grateful if we can -- looking at the timings and the entries for 02.00 and 03.00 on the left-hand side. In fact, it's the second and third from the left, but if you want to check the timings we can scroll up and see -- there's 02 and 03. Can we go down to the bottom to see what they record, please? I'm afraid it's side-on here.

I should say, for the paper enthusiasts, this is in the paper bundles but it's in pretty minute writing and you have to turn it sideways-on to see it.

Thank you. Mr Murphy's done it for us.

At 02.00, what does your entry say there? Can you see, Ms Letby?

LETBY: A chest drain's been inserted at 02.10, and saline bolus given at 02.50.

MYERS: Right.

MR JUSTICE GOSS: Sorry, what does it say? "Chest" and then what's the next --

LETBY: "Chest drain." A chest drain is inserted started at 02.10 and then a 10ml/kg saline bolus at 02.50 via the long line.

MR MYERS: All right. The chest drain is the third one that was fitted and that's by Dr Gibbs.

LETBY: Yes.

MYERS: The bolus actually has a timing there, 02.50.

LETBY: Yes.

MYERS: And under what time have you written "blood complete"?

If you were to look go right to the top of that column what's the time --

LETBY: Some time after 3 o'clock.

MYERS: Yes, all right. 3.24 is resus.

LETBY: Yes.

MYERS: We can see from those charts when it was that the blood transfusion was completed?

LETBY: Yes.

MYERS: Is there some sinister reason why in the nursing notes it says "transfusion at 02.00"?

LETBY: No, that's just an error on my part when I was writing the notes.

MYERS: Is there any mystery for anybody who wants to see on these charts where it says "blood completed"? Are they readily accessible?

LETBY: They are, yes. There's also readings on the right-hand side of this chart to show when the transfusion stopped and also on the observation chart as well. So many areas that the times are documented in, as well as my nursing note.

MYERS: Outside what you put in your nursing note, is there anything you're able to add as to the care for [Baby H] or what happened when she did have that desaturation at 3.22/3.24?

LETBY: No.

MYERS: Was she stabilised?

LETBY: After that?

MYERS: After that.

LETBY: Yes.

MYERS: Can we go to tile 296, please, Mr Murphy, next.

We're going to look at several tiles here and then a handful more that come a little later on.

8.49 on 26 September. So this is -- is this the same morning but later on?

LETBY: Yes.

MYERS: So would you still be on the unit at this time?

LETBY: No.

MYERS: Is this after your shift has finished?

LETBY: Yes, it is.

MYERS: It's a message from Yvonne Griffiths to you. It says:

"Hope you have a good sleep. I just want to commend you for all your hard work these last few nights. You composed yourself very well during a stressful situation. It's nice to see your confidence grow as you advance through your career."

And then can we go to tile 297, please. It's a message from you to [Nurse A] at 8.52 saying:

"Hope you have a good sleep. I just want to commend you for all your hard work these last few nights. You composed yourself very well during a stressful situation. It's nice to see your confidence grow as the advance through your career."

Then the next tile, 298, where you say:

"From Yvonne. What do I say to that?"

LETBY: Yes.

MYERS: What were you sending from Yvonne to [Nurse A]?

LETBY: I just replicated the message to [Nurse A].

MYERS: First of all, Yvonne Griffiths. Why would Yvonne Griffiths be sending a message like that to you, in what capacity would she be doing that?

LETBY: Yvonne Griffiths is part of the management team.

MYERS: You send a message to [Nurse A]. Is there any particular reason why you sent it to [Nurse A]?

LETBY: Because [Nurse A] had been on shift with me that previous night.

MYERS: Had there been any particular issue that that -- that made that relevant when it came to sending it to [Nurse A]?

LETBY: Yes, there had been some disagreement between Yvonne Griffiths and [Nurse A] regarding the baptism of [Baby H].

MYERS: Could you tell us a bit more about that so we and the jury can follow?

LETBY: Whenever a baby is critically unwell we offer a baptism to parents. That was something that Dr Gibbs offered to [Baby H]'s parents following her resuscitation. The parents wanted that to happen. As shift leader [Nurse A] sort of made arrangements to start organising that and Yvonne Griffiths didn't feel it was appropriate to be done at that time of night.

MYERS: Was there a reason why she thought it wasn't appropriate from what you saw taking place?

LETBY: She felt the unit was busy and [Baby H] had stabilised and therefore it wasn't something that needed to be done in the middle of the night and it was a discussion between the two of them as the two senior staff.

MYERS: Were you any part of that discussion?

LETBY: No.

MYERS: Let's just read on to what happened. If we look at tile 302. 302 is [Nurse A] to you, 09.01 that morning, 26th, you having sent Yvonne's message or forwarded it:

"Wow!!! Maybe she wasn't glaring at you just watching how you coped after I'd disagreed about the christening. She is absolutely right xx"

Tile 303. [Nurse A] to you, 09.04:

"Reply: Thank you for the message. It means a lot to me to hear you say that. Thank you for yours (and especially [Nurse A]'s) support over the last few nights. (It particularly means a lot of as several of the band 6 staff seem to think I'm not up to it. Do you mind if I show them this and tell them to shut the [excuse my language] fuck up?)"

That's from [Nurse A] to you.

LETBY: Yes.

MYERS: Pausing there, what was she recommending you do when she said that?

LETBY: That Yvonne's positive comments were shown to other members of staff.

MYERS: Right. Let's look at tile 304, which follows this, from [Nurse A] to you:

"Bits in brackets optional lol."

Tile 305:

"Feel a bit bad now. Maybe she's had a little think on way home."

Tile 306, you to [Nurse A], 09.04:

"Ha ha, love the bit in brackets, but may leave it out!"

Tile 307, [Nurse A] to you, 09.06:

"Yep, I think my disagreeing and saying that you were more than coping and also saying that you were choosing not to have [Baby H] due to possible lack of appropriate support may have made her think. I hope so cod [because] I'm still bouncing about a lot of things."

I'm going to ask you a few things about that, Ms Letby. Do you know what [Nurse A] was referring to, do you know what she's referring to when she says here:

"I think my disagreeing and saying that you were more than coping"?

Do you know what that means, you were more than coping? Do you understand what that related to?

LETBY: Yes, that I was managing [Baby H]'s care needs.

MYERS: "... and also saying that you were choosing not to have [Baby H] due to possible lack of appropriate support may have made her think."

LETBY: Yes, so by this time [Baby H] had a lot of chest drains which I was not overly familiar with, and I had voiced that to staff and that was why [Nurse A] was placed in the nursery with me, to care for [Baby H] between the two of us.

MYERS: But it says you were "choosing not to have [Baby H] due to possible lack of appropriate support"?

LETBY: Yes, I said I wasn't happy to have [Baby H] on my own.

MYERS: The prosecution say you wanted to kill her. Didn't you want her on her own so you could kill her?

LETBY: No.

MYERS: What were you asking for?

LETBY: I wanted support from a senior member of staff.

MYERS: All right. Let's read on then, please. Tile 308, [Nurse A] to you, 09.07:

"I haven't had a message lol. I think I've pissed her off big time."

Next message, tile 309, 09.07, you say to Yvonne:

"Thank you. That's really nice to hear as I gather you are aware of some of the not-so-positive comments that have been made recently regarding my role, which I have found quite upsetting. Our job is a pleasure to do and just hope I do the best for the babies and their families. Thank you to you and [Nurse A] for your support."

Pausing there, what are you referring to when you say:

"I gather you are aware of some of not-so-positive comments that have been made recently regarding my role which I have found quite upsetting"?

LETBY: Some of the existing staff members weren't overly happy with people like myself and Shelley Tomlins having babies in nursery 1.

MYERS: Can you explain what that's about or why that is?

LETBY: So quite often, when you're newly qualified and just done your ITU course, you're encouraged to go into nursery 1 to get experience and I think some of the other staff felt that they wanted that experience and that we had got that at the tertiary centres.

MYERS: Right. We'll move on then, please. Tile 310. From you to [Nurse A], 09.08 that morning:

"I've replied saying 'Nice to hear as been a few not-so-positive comments about my role that which I have found quite upsetting and thanks to you and [Nurse A] for supporting me'."

Then tile 311, please, 09.09, you to [Nurse A] again:

"I'm still frustrated/upset with what's gone on but don't think such rubbish nights and being tired help."

Pause there, what do you mean when you say you are still frustrated and upset with what's gone on? What is it that's gone on?

LETBY: So as I said before some of the comments that were being made about allocation.

MYERS: What do you mean "such rubbish nights", when you say "such rubbish nights"?

LETBY: I'd had very busy and difficult nights.

MYERS: Let's finish this off with tile 312 from [Nurse A] to you:

"Good reply as it's important they know about the bitchiness, which is all it is. Yes re tired. I know I overreacted to Yvonne telling me off but I still feel like crying about it now."

What you've just described, Ms Letby, what you described to us about comments or things being said, did that carry on, did it stop?

LETBY: About me specifically?

MYERS: Yes, the sort of things you were talking about in these messages. As we go on through 2015 into 2016, did it carry on?

LETBY: Not that I recall, no.

MYERS: Is it something which was particularly prominent at this time?

LETBY: Yes.

MR JUSTICE GOSS: So the comments about which you were complaining in these messages that you'd been having at this time, they didn't go on after this?

LETBY: No.

MR JUSTICE GOSS: Right. That's the question you're being asked, I think.

LETBY: Oh yes, sorry.

MR MYERS: I'm going to move then next to count 11, so I wonder if this is the right time to stop, my Lord.

MR JUSTICE GOSS: Yes, certainly.

2.05, please, members of the jury.

(In the absence of the jury)

MR JUSTICE GOSS: I know that the public aren't present, but the court is still sitting and the defendant is present.

Thank you, Mr Astbury, for the updated or the revised, corrected, agreed facts. I assume that copies of those are going to be distributed at some stage, which is --

MR ASTBURY: Yes, they are, as are copies of the indices for both bundles and also an index of the various images that we've seen, which have been prepared. The question was timing and I wasn't proposing to interrupt at this stage.

MR JUSTICE GOSS: I'll just leave it to the prosecution and defence to decide what is an appropriate time to do it, but I agree with you, it's not an appropriate time in the middle of this.

The other thing is the issue with Ms Moritz. Has that been resolved?

MR ASTBURY: Yes. She has helpfully reminded us we're going to discuss it over lunch.

MR JUSTICE GOSS: Thank you very much. I will now rise then, which will complete the sitting this morning.

(12.58 pm)

(The short adjournment)

(2.05 pm)

(In the presence of the jury)

MR MYERS: Ms Letby, we're going to turn next to the second event relating to [Baby H] and that's count 11 on our indictment, and that is an event that took place on 27 September 2015 at about 00.55.

That's a shift that you were working, and we can just have a look at the layout before we look at the events. That's tile 385, please, Mr Murphy -- sorry, this is the [Baby H] SoE.

Just take a moment to absorb the shift, Ms Letby, who's on. There's a lot of detail we're going through with all of these.

We see there's five members of nursing staff, including the shift leader Christopher Booth.

If we scroll down we can see the babies and the rooms. So there's 12 babies on this evening, and [Baby H] is in nursery 1 with Shelley Tomlins and you were in nursery 2 with two babies there.

LETBY: Yes.

MYERS: And also Christopher Booth is in there. In fact, there's an additional baby that came on to the unit during the course of the night, whose initials are LO.

LETBY: Yes.

MYERS: It's recorded at the back of the neonatal review, ladies and gentlemen, LO was transferred on to the unit at about 05.10, so that adds to the babies, but that comes after the event that we're looking at.

Do you have any particular recollection of what took place on this night involving [Baby H] outside the nursing notes?

LETBY: No.

MYERS: You weren't her designated nurse?

LETBY: No.

MYERS: And you're looking after two babies in nursery 2?

LETBY: That's right.

MYERS: The allegation relates to an event at 00.55 and just to see what that consists of, I'm going to ask if we can go to the nursing note that was actually made by Shelley Tomlins, who's the nurse who was looking after [Baby H], and that's at tile 436. If we go into that, please, Mr Murphy.

There is an amount of material to look at here that Nurse Tomlins has recorded and it's necessary, as with other cases, for me just to look at the material so we can acquaint ourselves with what had taken place before I ask you to say what you're able to say.

Nurse Tomlins' note was a number of notes made by Nurse Tomlins. I'm not going to deal with the content of all of them, but we can see the first note made is at 22.53 that night. Do you see that, Ms Letby?

LETBY: Yes.

MYERS: Starting from when the shift began at 20.00 hours and identifying that care was taken over for [Baby H]. I'm going to pick out events as we go through. If we look at the second paragraph, it says:

"Drain removal. Long line occluded frequently while bolussing, therefore 5ml of 5.8ml dose given. Around 20.30, [Baby H] had profound desaturation and brady, air entry no longer heard and capnography negative."

That means no colour change on the capnograph, as we know now:

"Therefore ETT removed and doctors crash bleeped. [Baby H] bagged up well with jaw thrust. New ETT sited by Dr Neame on second attempt."

It goes on to deal with what happened there. That is an event at 20.30, not something attributed to you, not something you're blamed for, Ms Letby.

LETBY: No.

MYERS: But that's a profound desaturation and bradycardia at 20.30. If we scroll down, please, same nurse, same nursery, same baby. Shelley Tomlins at 23.09 that night with an entry now for 21.45, again not the incident on the indictment that's brought against you, but 21.45:

"Desaturation to 40% despite good air entry and positive capnography. ETT suctioned quickly with thick bloodstained secretions noted. [Baby H] recovered quickly after. Neopuffed effectively via tube."

Were you aware of anything to do with that?

LETBY: No.

MYERS: Go down to the next entry, an entry made at 02.52 by Shelley Tomlins, now in the morning of 27 September:

"00.55. Profound desaturation to 40% despite equal bilateral air entry and..."

(Pause)

Can we go to tile 442 then, please?

There we are, carrying on, on the top left. We've just seen the entry for 00.55:

"Profound desaturation to 40% despite equal bilateral air entry..."

And we carry on here:

"... and positive capnography. ET suction yielded nil secretions. [Baby H] then went bradycardic at 01.07 to 40bpm and required chest compressions and add written at 01.08. Saline bolus given at 01.12 hours. Transillumination showed no further pneumothorax. Chest X-ray [perhaps that should be] ordered and carried out, including one left lateral angle."

This incident is one for which you were blamed, Ms Letby. Did you have any involvement with this?

LETBY: No.

MYERS: Did you have any awareness of this happening at the time it happened from your recollection?

LETBY: Not from memory, no.

MYERS: Did you have anything to do with it?

LETBY: No.

MYERS: Right. If we carry on and scroll down, please, the next entry at 03.49, now at 03.30, an incident recorded, again not the one alleged against you:

"Profound desaturation to 60s, again requiring Neopuffing with no known cause for desat. Air entry still heard bilaterally and capnography positive. Copious amounts of secretions yielded orally, pink-tinged. Small amounts of ET secretions gained, again pink-tinged."

It goes on to say:

"Heart rate mainly normal during desat, recovered slowly."

References to urine output and transfer to Arrowe Park.

We can see at 05.58 an entry by Nurse Tomlins saying:

"[Baby H] moved to transport incubator without issue."

The notes by Nurse Tomlins identify events at 20.30, profound desaturation and bradycardia, an event at 21.45, desaturation, an event at 00.55, desaturation, and resuscitative measures required, and an event at 03.30.

Did you have any particular awareness of any of these on this evening so far as you can remember?

LETBY: No.

MYERS: Are you able to account for what happened on any of them?

LETBY: No.

MYERS: Do you recall what involvement, if any, you had with [Baby H] on this particular night independently?

LETBY: I don't recall any from memory.

MYERS: Let's just have a look at the neonatal review. I just ask us to keep in mind in particular the time of that third event at 00.55, that's the one you're blamed for.

Let's look at the neonatal review. You'll find this easiest in paper, but Mr Murphy can help us on screen and I'm going to ask if we could go to page 5, please.

There will be a bit of moving between the review and a few tiles here, ladies and gentlemen, but bear with it, please.

Just looking at what we have from lines, perhaps, 71 to the event at line 95, and we can home in on this area, thank you, we know that blue relates to you, Ms Letby, and the yellow squares relate to [Baby H].

LETBY: Yes.

MYERS: The event that we're dealing with on count 11 is down at the bottom, line 95. Do you see that? It's rather small, 03.22.

LETBY: Is this for the previous shift?

MYERS: Sorry, my mistake. There's two of them, I should have asked for the second. My apologies, Mr Murphy. It's the second neonatal review for [Baby H]. We've got two as far as I can see. This is page 5 of the second one. Event 11, it's called.

(Pause)

What we're looking at, Ms Letby, is the extent of your involvement recorded on this review with [Baby H] prior to the event that's alleged against you at 00.55.

(Pause)

If we've got it in paper, ladies and gentlemen, it might be lost in the machinery. Mr Murphy, don't worry about that. Maybe we can get the actual exhibit and let's look at the neonatal review for event 11.

(Pause)

MR JUSTICE GOSS: Have you got it? It's the second one for [Baby H]. It's event 10, 11 pages, so it's got page 1 to 11 at the bottom. I don't know whether you were flagging it, tabbing it as I was. I've got flag 10 as my flag.

MR MYERS: I think we're there, thank you, my Lord.

Page 5 of 11, ladies and gentlemen. I just want to look at your involvement, so far as it's recorded here, with [Baby H] on this evening before the event alleged against you at 00.55. As it happens, we go to page 5 for where you first appear so far as [Baby H] is concerned. Can we go to line 71?

It's there, thank you, Mr Murphy.

Line 71. In fact, if we enlarge lines 71 to 75, this will be the easiest. 71 to 74. There we are.

Can you assist us with what we see at 22.12, please, looking at this, Ms Letby? Line 71.

LETBY: Yes, myself and Shelley Tomlins have given a medication to [Baby H].

MYERS: So you and Shelley Tomlins giving medication to [Baby H]?

LETBY: Yes.

MYERS: That's timed at 22.12. I just wonder if we can, hoping we don't lose the neonatal review so we can go back to it in a moment, go to tile 405 to see what that was.

One moment.

(Pause)

I don't know what's happening with the references here but I've got, for tile 405, exhibit 12603. It's the prescription and it's J number 12603 if that helps, Mr Murphy.

(Pause)

Tile 414, thank you very much. Can we go into that, please, and have a look at what that shows us? So what medication are we dealing with here?

LETBY: Benzylpenicillin.

MYERS: If we look further down the page, a bit further down to see who did what. 22.12. Can you see the first entry?

LETBY: Yes.

MYERS: Can you tell us what it is that's happened there?

LETBY: So that's myself and Shelley Tomlins signing to say that benzylpenicillin was given at 22.12 to [Baby H].

MYERS: Where it says looking across from there, "Filed 22.13", what's that referring to on the right-hand side?

LETBY: So that's the time that has been -- that we've both inputted our signatures on to the system.

MYERS: So when we look in the neonatal review on line 71, I can see we've all got it in front of us, and we can see there "Prescription 22.12"?

LETBY: That's the time medication was actually given but 22.13 is the time that we've both finished on the computer.

MYERS: That's why on line 73 we have a pair that goes with that. It says:

"E-prescription administration history update made on computer."

LETBY: Yes.

MYERS: So although there are four entries, there are actually two medications being delivered; is that right?

LETBY: That's right, yes.

MYERS: So that was the first of them, 22.12.

Line 72 is the second. What does that tell us?

LETBY: Same again: a medication was administered at 22.13.

MYERS: And on this occasion can we look at tile 415. Can we go into that to see what the medication is. What was being administered at 22.12?

LETBY: This is a sodium chloride flush to go after the antibiotics. We always use a flush after each medication.

MYERS: If we can scroll down, we'll see where that's recorded.

Can you see there, Ms Letby, where the flush you're describing is actually recorded?

LETBY: Yes.

MYERS: Where is that and tell us what it shows?

LETBY: It's where the arrow is now, so it's 22.13, 2ml of sodium chloride was given by myself and Shelley Tomlins.

MYERS: And recorded also at 22.13?

LETBY: Yes.

MYERS: So that's the item in line 72 on the neonatal review?

LETBY: Yes.

MYERS: And the update is two lines below that where it's been filed?

LETBY: Yes.

MYERS: Is that something that would happen as a standard, that after the benzympenicillin there would be a sodium chloride flush?

LETBY: Yes, so after any medication that we give it is always accompanied by a flush of sodium chloride.

MYERS: That's at 22.12 and 22.13; yes?

LETBY: Yes.

MYERS: We know we're looking at an event at 00.55. So let's move forwards to where you next have involvement with [Baby H]. Can you tell us where that is on the chart, please?

LETBY: 22.38.

MYERS: Thank you. That's at line 82?

LETBY: Yes.

MYERS: What happens there?

LETBY: Again, myself and Shelley have signed for a medication.

MYERS: All right. Can we look at tile 420 to see what that medication is? Perhaps first of all just confirm the timing is right. Can we scroll down to see who does what on the relevant timing, Mr Murphy?

22.38. Who's done what here?

LETBY: At 22.38, a morphine infusion has been started by myself and Nurse Tomlins. It's been started at 22.38.

MYERS: So line 82 on the neonatal review, 22.38, it's got you and Nurse Tomlins recorded?

LETBY: Yes.

MYERS: What have you done at 22.38 looking at the item we've just put on the screen?

LETBY: Started a morphine infusion.

MYERS: And at line 83, what's happening there?

LETBY: That's when it's been fully filed on to the computer system.

MYERS: Does that dovetail with the 22.39 --

LETBY: The 22.39 on the right-hand side, yes.

MYERS: So that's the involvement with you and [Baby H] at 22.39.

We can take that tile down, thanks very much, Mr Murphy.

Sorry about the long trip round tile 414.

If we just come back to the neonatal review, if we just move forwards, we can see when you next are recorded with any involvement with [Baby H]. Can we go over the page to page 6, please?

Lines 108 and 109. Who are you involved with then?

I don't need the full name.

LETBY: This is with another baby on the unit.

MYERS: EM is one of your designated babies in fact?

LETBY: Yes.

MYERS: That's at 00.00. The event at 00.55, are you anywhere near to [Baby H] when that happened as far as you can recall?

LETBY: No.

MYERS: Let's carry on to see where you do next have contact with [Baby H]. Over to page 7. This has times between 01.44 through to 02.52. Any involvement with [Baby H] during this period?

LETBY: No.

MYERS: Then finally over to page 8, please. We know, just remind us, the fourth event Shelley Tomlins described is a desaturation at 03.30. Do you remember seeing that final event?

LETBY: Yes.

MYERS: That's not the one that's on the indictment against you, but there was an event at 3.30. Did you have any involvement with [Baby H] after that?

LETBY: Yes, post that event, yes.

MYERS: Where do we see that on the chart?

LETBY: From line 184.

MYERS: 184. In fact, 184, 185 and 186?

LETBY: Yes.

MYERS: So there's medications given at that point?

LETBY: Yes.

MYERS: Who do you do that with?

LETBY: Shelley Tomlins.

MYERS: Do you recall why it was necessary to give those medications?

LETBY: I don't know what the medications were without looking.

MYERS: I'm not going to go into the detail there insofar as this isn't something that's alleged against you. But between helping with those prescriptions leading up to 22.39 and then helping again after that event at 03.41, did you have any involvement with [Baby H] at all?

LETBY: No.

MYERS: Thank you. I'm going to move forwards in time, Ms Letby, so far as the events concerning you and this indictment are concerned, to [Baby I]. I'm going to assist again just by summarising what we're dealing with when we come to [Baby I], but then we'll take our time to go through each of the incidents in order, putting the neonatal review to one side.

[Baby I] appears on our indictment at count 12.

The jury will recall, as will you, Ms Letby, that there are a number of different events which all come under that particular allegation.

LETBY: Yes.

MYERS: You're aware of that, aren't you?

LETBY: Yes.

MYERS: So what we'll do to start with is just, as we've done elsewhere, remind ourselves of the basic facts relating to [Baby I] and the dates that we're dealing with and then we'll look in more detail as we go through the particular events. Do you understand?

LETBY: Yes.

MYERS: Thank you. So [Baby I]. Let's start with tile 2 from the [Baby I] sequence number 1.

We can see [Baby I], born on 7 August 2015 at 20.47 at Liverpool Women's Hospital. 27 weeks' gestation at birth. 970 grams.

She was born at Liverpool Women's Hospital, she was transferred to the Countess of Chester on 18 August.

Can we put up tile 4 from this sequence, please, Mr Murphy? 18 August she was transferred. Go behind that. This is some notes made by Dr Sally Ogden. Can we scroll down the page if you would, please?

Move to tile 5, please. And go into that.

Thank you. Dr Ogden deals with the notes when [Baby I] first comes into the unit, just to remind ourselves.

Active problems, as they're described at this point: preterm, RDS. Remind us, what's RDS?

LETBY: Respiratory distress syndrome.

MYERS: "Est feeds"?

LETBY: That's establishing feeds, milk feeds.

MYERS: Then "jaundice and suspected sepsis"?

LETBY: Yes.

MYERS: This is on arrival at the Countess of Chester, 11 days after birth. That's at the start to the middle of August. I'm just going to remind us of the dates of the events we've dealt with and we can go through some of the detail.

First two dates that we looked at, which aren't actually on the sequences, are 23 August and then the 5th to 6 September. Just to remind us, 23 August is an event regarded as a suspicious event by Drs Evans, Bohin and Marnerides. That's linked to a radiograph showing air in [Baby I]'s abdomen area. That was 23 August.

The 5th to 6 September 2015 related to a series of desaturations during which [Baby I]'s condition deteriorated and she was then transferred back to Liverpool Women's Hospital on 6 September. Those don't form part of the allegation against you.

LETBY: No.

MYERS: But they're background, not in a chronology that we have on the sequence of events, but they're part of the chronology overall.

LETBY: Yes.

MYERS: [Baby I] returned to the Countess of Chester on 13 September 2015, and then the first event we are dealing with in terms of allegations against you, Ms Letby, is on 30 September 2015. We'll go back through those just to reassure everyone in a bit more detail.

30 September 2015, when, after a feed at 16.00 hours, there was vomiting, desaturation and bradycardia, and then a few hours later, at 19.30 hours, further apnoea, desaturation and bradycardia. So that's the first event. There's two parts to that really, and that's on 30 September.

The second event is on 13 October 2015 and this is an event when there was a deterioration of [Baby I] in nursery 2 at 03.20 in the morning, when Ashleigh Hudson had previously been caring for her, but her condition deteriorated from 03.20. In fact, just to

assist everybody, that's the event where there's the issue about what the lighting was in the nursery at the time that happened. We'll come back to that.

The third of the four events, again one with two parts to it, is on 14 October, so the following morning when, at 05.00, [Baby I] was identified to have a distended abdomen with discolouration on one side of it and she deteriorated and she required ventilation and resuscitation.

Then 2.5 hours later at 07.30, further desaturation, bradycardia and discolouration on handover. In fact, [Baby I] then continued to deteriorate and she went to Arrowe Park Hospital on 15 October and she remained there until the 17th, before coming back to the Countess of Chester.

Then we come to the final event, again another event with two parts to it, on the night of 22 October into 23 October 2015. Shortly before midnight, 23.57, there was a desaturation and [Baby I] required CPR and an abdominal X-ray was taken at about 00.23, but the first part of this event was a desaturation at 23.57. That was followed at 01.06 by another desaturation and there was CPR and, sadly, [Baby I] did not recover from that.

There's a lot there and we'll go through it step by step. Just to help, the key dates that we've looked at: 23 August, we identified, the 5th and 6 September, and then the four events, event 1 on 30 September, event 2 on 13 October, event 3 on the 14th, and event 4 on the 22nd into 23 October.

Before we look at the particulars of any of those, Ms Letby, do you have general recollection of [Baby I] during her time at the Countess of Chester Hospital?

LETBY: Yes.

MYERS: And how would you describe her in general terms during her stay there?

LETBY: [Baby I] was a baby that was with us for many months and we got to know her and her family well during that time.

[Baby I] had a number of ongoing complex issues and I remember her being regularly transferred to and from the unit for various reasons, mostly to do with her abdomen.

MYERS: What was it about her abdomen?

LETBY: For [Baby I] it was always distended.

MYERS: So we're looking at in this case certain incidences when it was found to have distended. Are those the only times her abdomen distended?

LETBY: No, [Baby I]'s abdomen was always more distended than normal and then she would have episodes where that would increase.

MYERS: Did you care for her on a number of occasions?

LETBY: Yes.

MYERS: She was at the Countess of Chester on and off for a period of months, wasn't she?

LETBY: Yes.

MYERS: Did you care for her on more occasions than the times we've identified in the events here?

LETBY: Yes.

MYERS: 23 August, I want to ask you about this, that's been identified as a suspicious event. I wonder if we could just put up the radiograph. It's in the additions and it's exhibit 13960.

This is a radiograph taken on 23 August, Ms Letby.

You've been following this in the evidence, haven't you?

LETBY: Yes.

MYERS: 23.02. If we just look at the commentary that goes with that, which is at page 13807, if we just scroll down and enlarge that. The description -- it's just the first part of it I'm going to -- is:

"There is non-specific gaseous distension of the abdomen, which is suggestive of developing necrotising enterocolitis."

Could we look at the shift pattern at page 33224, please, Mr Murphy, which is also behind tab 23 in jury bundle 2, ladies and gentlemen, but it'll appear on the screen. It's the shift pattern, page 33224.

23 August has been identified as an event where some of the experts have given the opinion harm was done or attempted. If we look into August, can we look in the area leading up to the 23rd? Were you at work at that time?

LETBY: No, I wasn't.

MYERS: If we want to see who was caring for [Baby I], can we put up exhibit 13807. I'm just interested in identifying what we have here. 23/08/15. So at 09.35 that morning, there's an entry by [Initials of Nurse C]. Who is [Initials of Nurse C]?

LETBY: [Nurse C].

MYERS: If we scroll down, the next entry is at 17.19. Again who's put that entry in?

LETBY: [Nurse C].

MYERS: And an entry at 18.27 on 23 August; who's put that in?

LETBY: That's also by [Nurse C].

MYERS: Right. And that continues across the page. As a rule, who is the nurse who is responsible for putting the entries in about a particular baby?

LETBY: The designated nurse.

MYERS: I make it plain, we're not making any particular suggestions about [Nurse C]. But from looking at this, who's caring for [Baby I] over 23 August 2015 --

LETBY: [Nurse C].

MYERS: -- in that day?

LETBY: It's her throughout the shift.

MYERS: All right, thank you.

I'm going to move forwards then to the first of the events which form part of what's alleged against you, Ms Letby. So this is 30 September now. Just to remind us all, we're looking at things that happened at 16.00 that day followed by a couple of hours later at 19.30, so 30 September. Let's have a look at the layout.

That's at tiles 43 and 44, please, Mr Murphy.

Tile 43. Shift leader, [Nurse B].

You're the designated nurse for [Baby I] on this day?

LETBY: Yes.

MYERS: Is this a day shift that we're dealing with?

LETBY: Yes.

MYERS: Can we scroll over the page to tile 44. Which nursery were you working in that day, Ms Letby?

LETBY: Nursery 3.

MYERS: Who were you looking after?

LETBY: I was looking after [Baby I] and two other babies.

MYERS: Do you have recall or detailed recall of the events of 30 September?

LETBY: I have some memory, but not in great detail.

MYERS: Right. So far as [Baby I]'s deterioration at 16.00 is concerned, and we'll look at the detail in a moment, did you do anything to cause that to happen?

LETBY: No.

MYERS: Would you?

LETBY: No.

MYERS: And the second part of this event, which is at 19.30, around the time of handover, do you remember who you were handing over [Baby I] to at that time?

LETBY: Yes, it was Nurse Butterworth.

MYERS: Do you recall what happened during handover as you handed over to Nurse Butterworth that evening?

LETBY: Yes.

MYERS: Can you describe it to us, before we go to the notes, from your memory?

LETBY: We were both at the cot side, I was giving a handover to Nurse Butterworth, and [Baby I] became apnoeic and we began Neopuffing and we could see her abdomen was distending as we were Neopuffing.

MYERS: To be clear about that, was her abdomen, so far as you could see, distended before you were Neopuffing?

LETBY: No.

MYERS: So could you describe what happened?

LETBY: As we were Neopuffing her, we could visibly see that her stomach was distending and we aspirated her NG tube and got a lot of air back and her abdomen returned to what was normal for [Baby I].

MYERS: The air that you got back, was that before or after you had used the Neopuff?

LETBY: After.

MYERS: And there are the notes that you made -- you were the designated nurse. So can we go to tile 61, please, in this sequence.

The actual event comes in the tile that follows, but we have a note, first of all, of 30 September at 13.36:

"Written for care given from 08.00 to present."

We'll have a look through that. Would you read out for us what it says in fact?

LETBY: "Emergency equipment checked. Fluids calculated. [Baby I] nursed on a hot cot. Initially set at 36.5 degrees. [Baby I]'s temperature 36.2 this morning. Hot cot increased. Temperature now 36.4. Hot cot currently set at 38 degrees. 3x8 feeds of expressed breast milk, two bottles to one NG tube. Abdomen appears full and slightly distended. Soft to touch. [Baby I] straining ++. Bowels have been opened. Mum feels it is more distended to yesterday and that [Baby I] is quiet. Appears generally pale. Not on monitor but nil increased work of breathing. Handling well and waking for feeds. Doctors asked to review. To continue with current plan."

MYERS: Any particular problems up to this point?

LETBY: Yes, [Baby I]'s got an ongoing low temperature.

MYERS: And what about the situation with her abdomen?

LETBY: So her abdomen is distended but it's soft to touch.

MYERS: Right.

LETBY: She's had her bowels opened.

MYERS: Let's carry on with the notes you make at tile 62 next.

Now we're at 19.31 and we're dealing therefore with the time after the incident or certainly the first incident.

So could you read on, please?

LETBY: "Reviewed by doctors at 15.00 as [Baby I] appeared mottled in colour with distended abdomen and more prominent veins. Advised to continue. Temperature within normal range with hot cot at 38 degrees. Full monitoring recommenced within normal range. Bowels opened, yellow seedy stool."

MYERS: I will just ask you to pause there for a moment:

"Reviewed by doctors at 15.00 as [Baby I] appeared mottled in colour."

Do you have any recollection as to how it would have come about that she was reviewed by the doctors at that time?

LETBY: I asked for her to be reviewed.

MYERS: Why did you ask for that?

LETBY: Because she was -- she became mottled and her abdomen looked more distended than it had prior to -- earlier in the shift.

MYERS: That's at 15.00?

LETBY: Yes.

MYERS: Okay. Let's carry on. You got as far as yellow seedy stool.

LETBY: "Did not wake for feed at 16.00, therefore NG tube feed given, expressed breast milk with fortifier. At 16.30 [Baby I] had a large vomit from mouth and nose ++. Suction given. Became apnoeic with bradycardia and desaturation to 30s. Help summoned and IPPV given for approximately 3 minutes in 100% oxygen to recover. Doctors were crash called."

MYERS: All right, I'm going to ask you to stop there. We see she was then transferred to nursery 1. Were you there throughout the events we're looking at here?

LETBY: No.

MYERS: What else were you doing?

LETBY: I was looking after the other babies in the nursery.

MYERS: That's probably my fault for not being clear. When I said were you there, first of all, were you in the same nursery as far as you can remember?

LETBY: Yes.

MYERS: But you then go on to say [Baby I] wasn't the only baby you were looking after in there.

LETBY: No.

MYERS: So there were three babies in there?

LETBY: Yes.

MYERS: And do you recall the vomit that [Baby I] had at 16.30?

Outside what's written in the notes do you have any recollection of it?

LETBY: Yes, a little bit.

MYERS: Do you recall what happened?

LETBY: I just remember she had a vomit and her monitor -- that she vomited and I went over to her and she needed Neopuffing briefly.

MYERS: All right. Let's just read through if you would, please, to the end of this entry.

LETBY: "Transferred to nursery 1 and placed in an incubator for closer observation. Nil by mouth. NG tube on free drainage. Blood gas as charted. Cannula inserted but tissue during saline bolus. 5ml given. 10% glucose infusion stopped. Has not had antibiotics. Reg [Dr A] currently trying to site IV access. Colour appears pale but improved from earlier in shift. Abdomen still appears full and distended, veins more prominent. Not further vomits."

MYERS: It says "not further" but "no further vomits".

LETBY: I think it means "no":

"Responsive but quiet on handling. Self-correcting desat to 80s. May require blood transfusion. Chest and abdominal X-ray carried out, awaiting review by doctors."

MYERS: Thank you. Where it says -- where you put, "Self-correcting desaturations to 80s", what is it that's happened for you to write that entry, what's taken place?

LETBY: So [Baby I]'s been having desaturations, going down to the 80s, but not needed interventions, she has self-corrected as we call it.

MYERS: And you have recorded that here?

LETBY: Yes.

MYERS: And can people read this if they wish to as they go through the shifts?

LETBY: Yes, they'd be encouraged to, yes.

MYERS: So people would read back to this?

LETBY: Yes.

MYERS: Is there anything unusual in there being self-correcting -- letting desaturations self-correct when they drop to the 80s?

LETBY: No.

MYERS: Would you necessarily raise the alarm if saturations dropped to the 80s?

LETBY: Raise the alarm to somebody else?

MYERS: To call for help.

LETBY: No.

MYERS: So when do you know what to do? So when do you know to let it be self-correcting and when is it not self-correcting? Just so you can explain to us all.

LETBY: You have to give the baby time, so depending on their situation, you would give them a period of time to see if they self-correct, which most babies do. Whether that's sort of 30 seconds to a minute... If they don't respond, then that's when you would intervene. Here [Baby I] is having frequent desaturations to the 80s, but she's bringing herself up, she's not needing any intervention.

MYERS: Right.

LETBY: So it's important to document that she's desaturating but she's recovering herself.

MYERS: Thank you. If we could scroll down, Mr Murphy. There are two further entries that are relevant to this event, this pair

of events. So we have all of it, Ms Letby, we've got under the section "family communication", it says 30 September 2015 at 13.48:

"Mummy visiting. Carrying out feeds and cares."

Can you help us with what we have next at 19.32?

LETBY: "Mummy present when reviewed by doctors. Had left unit when [Baby I] had large vomit and transferred to nursery 1. Staff Nurse Taylor telephoned mum to update her and she is currently visiting. Up to date with plan of care and understands needs for current interventions."

MYERS: We're going to move next to the second part of this event which is what happened at 19.30. Do you remember whether [Baby I]'s mum was there at 19.30? Can you see here it says at about 19.32:

"... Taylor telephoned mum to update her and she is currently visiting"?

LETBY: Yes.

MYERS: Do you recall whether [Baby I]'s mother was present when we come to what happened at 19.30?

LETBY: I couldn't say definitively because some parents do leave during handover, so it would depend at what point we were up to.

MYERS: Let's have look at your note then made at 20.26, which deals with the second part of this event. Could you read it for us since you are reading this out for us, Ms Letby.

LETBY: "Peripheral line sited and 10ml per kilogram saline bolus given. 10% glucose commenced. At 19.30 [Baby I] became apnoeic, abdomen distended ++ and firm. Bradycardia and desaturation followed. SHO in attendance and registrar crash called. IPPV given, seal obtained but unable to gain chest movement. Air ++ aspirated from NG tube and chest movement and observations improved. [Baby I] is now very pale and quiet."

MYERS: Where we've got, "IPPV given, seal obtained but unable to gain chest movement", what is that describing, IPPV?

LETBY: IPPV is Neopuffing. Obtaining a seal means that we've got the mask around the mouth and nose, we've got a good seal, but we're not getting any chest movement.

MYERS: Was air being --

LETBY: Yes.

MYERS: -- put in --

LETBY: Yes.

MYERS: -- but the chest wasn't moving?

LETBY: Yes.

MYERS: Where was the air going then?

LETBY: It would have been going into her abdomen.

MYERS: And on your note is the air aspirated before or after the Neopuff is used?

LETBY: After.

MYERS: Let's have a look at what Nurse Butterworth said about the same incident and that's at tile 94, please, Mr Murphy. It's the first part of the entry I'm going to go to of this one and then the one that follows.

30 September, 22.35:

"For care commenced at 20.00 hours. All safety checks completed and satisfactory. During handover [Baby I]'s abdo had become more distended and hard. She had become apnoeic and bradycardic and sats had dropped. IPPV given and despite a good seal with Neopuff, there was still no chest movement. Aspirated NGT air +++ and 2ml of milk obtained. Eventually got chest movement and sats and heart rate normalised."

She goes on to deal with the medical support that was given after that. But does that describe the order in which events happened so far as you recall it, Ms Letby?

LETBY: Yes.

MYERS: All right. I'm going to ask you about some messages.

Is there anything you wish to add to what happened in the second part of this event at 17.30 -- 19.30 at handover or anything you can add?

LETBY: No, it's just -- I remember that and she recovered well afterwards.

MYERS: Right. Could we go, please, to tile 164? I want to look at some messages from 164 to 171 which are later that day or

into that night. We've been looking at events at 16.00 and 19.30.

23.00 that day or that night, from Jennifer Jones-Key to you:

"Oh, it's just [Nurse A] annoyed everybody last night going on about how amazing you are doing so many swaps and how naughty you weren't taken off today."

I'm going to read through these and then I'll ask you some questions as they arise. Can we go to tile 165, please, 3 minutes later, you replying to Jennifer Jones-Key:

"Oh was she? Kinda nice to hear something positive though as been a few not-so-nice comments."

Pausing there, you'll see this is 30 September.

Earlier we were looking at messages round about the time of events concerning [Baby H] on the 26th and 27 September.

LETBY: Yes.

MYERS: What are you talking about here where you talk about "there's been a few not-so-nice comments"?

LETBY: As I mentioned earlier, it was about some staff members feeling that myself and Shelley shouldn't be in nursery 1, that they needed the experience more than we did.

MYERS: All right. Can we go to tile 166, please, the next message in this sequence. A minute and a half later, Jennifer Jones-Key to you:

"It wasn't for us and pissed people off. I've done loads of swaps and extras. It was more the fifth time she said it. Why won't not nice comments [as read]?"

And we go to your response at 167:

"Everyone pulling their weight. I think she's just sticking up for me as knows I've had some rubbish said about me."

Tile 168, please, from Jennifer to you, 23.14:

"No, she's just sticking up for her friends and winding everybody else up. Shouldn't have said anything."

This is referring back to things [Nurse A] had apparently been saying. Do you know what this refers to?

LETBY: I'm not sure, no. [Nurse A] had obviously had a conversation with Jennifer, but not while I was there.

MYERS: Tile 169, please. It's less than a couple of minutes alter from you to Jennifer Jones-Key:

"I can't speak for [Nurse A] and I wasn't there. We've all been working hard."

Tile 170, please:

"That's half the problem, everyone tired."

We'll stop at that. Thank you.

When you're talking about, "That's half the problem, everyone tired", what was the state of affairs on the unit in terms of why everyone was tired?

LETBY: A lot of the staff were very, very drained physically and emotionally and the unit was very busy, lots of people were doing additional shifts and changing shifts at the last minute and it did start to have an effect on everybody.

MYERS: Right. Did you have, bearing in mind the BAPM guidelines, sufficient staff to cover all the types of babies you had at the acuity they were at?

LETBY: Not at all times, no.

MR MYERS: We'll move to the second event relating to [Baby I]. This may be a good time to pause, my Lord.

MR JUSTICE GOSS: If that suits you.

MR MYERS: Yes, thank you.

MR JUSTICE GOSS: It seems to be -- some time between 10 and 15 minutes. When you're ready, we'll begin. Thank you very much.

(3.00 pm)

(A short break)

(3.13 pm)

MR MYERS: Ms Letby, we'll turn now to the second event relating to [Baby I], and it is one event on this event, and it's what happened at about 3.20 in the morning of 13 October when Ashleigh Hudson had been caring for [Baby I] and then there was an event when both of you were at the nursery.

We'll start with what the notes say, Nurse Hudson's note and your note, and then we'll go through events as you recall them and the evidence relating to that. So with Mr Murphy's assistance can we go, please, to tile 48, which is the first, in fact, of the tiles -- we'll go into that, please.

It's that section, not all of it, but a large part of it. I'll deal with this, Ms Letby. This is a note by Ashleigh Hudson who's dealing with these events.

We'll look at the notes and then get down to the detail you can help us with.

05.04, 13 October. Nurse Hudson was the designated nurse for [Baby I]:

"Written for care taken over at 19.45 hours. Emergency equipment, alarm limits and feeds checked. Self-ventilating in air on handover. On a cot hot set at 36 degrees C. Temp 36.5C. Demanding and tolerating four-hourly feeds. Feeding well, alert when awake. Pale pink in colour but well perfused."

Then with regard to the event:

"03.22. When in the nursery, Neonatal Nurse Lucy Letby noticed that [Baby I] looked quite pale. When turning the light on for closer examination we found [Baby I] to be very pale in colour and not moving. Apnoea alarm in situ, had not sounded. Breathing was shallow and RR appeared low."

Is that likely to mean respiratory rate?

LETBY: Yes.

MYERS: "Philips monitoring commenced and Senior Neonatal Nurse Laura Eagles and paed's [paediatric] SHO Ekaterina Clegg shouted to assist. 30% Neopuff O2 [oxygen] commenced while Philips monitor establishing a trace."

It goes on to say:

"Philips monitor SaO2 [that's oxygen concentration] 53%. HR in 50s. Confirmed via stethoscope. Chest compressions commenced at 03.25. No HR heard. Paediatric Registrar Matthew Neame contacted via bleep. On arrival, Dr Neame took over the airway from L Letby and I gave breaths via Neopuff."

And the note then goes on to deal with care that was given. That's the note dealing with the deterioration made by Nurse

Hudson and can we also look at the note you made before we turn to your evidence on this and that's at tile 47, thank you.

Go down to 13 October, 07.59. Can you just read through your note for us, please, Ms Letby? It starts at the bottom of the page and finishes at the top of the other side of the page. We won't read all of it, but just to do with the collapse.

LETBY: "[Baby I] noted to be pale in cot by myself at 03.20. Staff Nurse Hudson present. Apnoea alarm in situ and had not sounded. On examination, [Baby I] centrally white, minimal shallow breaths followed by gasping observed. Ventilation breaths given via Neopuff and further help summoned. Full resuscitation commenced as documented in medical notes."

MYERS: We'll stop there because then it goes on to the ventilation and the steps taken to give [Baby I] the support she required.

So let's start with who was doing what on this particular evening. Can we look at tiles 30 and 31.

This is from the [Baby I] event 2 sequence. We see who's on duty, Ms Letby. The shift leader is Caroline Oakley. The designated nurse for [Baby I] is Ashleigh Hudson. You're on duty, so is Laura Eagles and so is Janet Cox. Let's have a look at who was where so far as the nurseries and the babies are concerned. So where are you when we look on this, Ms Letby?

LETBY: I'm in nursery 1.

MYERS: You were designated a baby in that nursery; is that right?

LETBY: Yes.

MYERS: We see that Ashleigh Hudson is designated three babies in nursery 2; is that correct?

LETBY: Yes.

MYERS: And [Baby I] was one of them. Can you -- first of all, did you have any involvement with [Baby I] prior to this event at 3.20?

LETBY: Not that I can recall from memory.

MYERS: Right. Were you asked to look after [Baby I] at any point before this event?

LETBY: No.

MYERS: And in particular, were you asked to look after [Baby I] any time round about 3 o'clock on the 13th?

LETBY: No.

MYERS: 3 o'clock in the morning. So you describe to us then the events that lead up to your involvement with this and the involvement itself, please.

LETBY: I recall myself and Nurse Hudson going to nursery 2 together and at some point, when I had just arrived, I noticed that [Baby I] looked pale in the cot.

MYERS: Right. Do you recall where you had been before you went to nursery 2 with Ashley Hudson?

LETBY: No, I don't.

MYERS: Or what you'd been doing?

LETBY: No.

MYERS: Or any idea where Ashleigh Hudson had been?

LETBY: No.

MYERS: You recall going to nursery 2?

LETBY: Yes.

MYERS: When you got there, who was doing what between you and Ashleigh?

LETBY: Ashleigh was doing something on the worktop, I can't remember what specifically, but she was stood with her back to the cot, doing something on the worktop and I was stood in the doorway talking to Ashleigh.

MYERS: Do you have any recollection of Ashleigh being involved in preparing a milk feed?

LETBY: From memory now, I couldn't say, but she was doing something at the workbench, yes.

MYERS: What was the illumination level like in that room when you first went to it with Ashley?

LETBY: I can't recall specifically what lights were on where.

I know I could see clearly enough that [Baby I] was pale in the cot. The room is lit from the corridor so there's a big large window, as we have seen in nursery 2. [Baby I] was in front of that window. There was also light coming in from the doorway and at no point is any nursery in complete darkness.

MYERS: Why would you never have a nursery in complete darkness?

LETBY: The only time we'd ever have that is in nursery 4 for babies who are preparing to go home. Otherwise we have lights on for safety -- we need to be able to see the babies.

MYERS: And why do you --

LETBY: And it is a high dependency room with three babies in it, so it's important that we're able to see them visually.

MYERS: What do you need to be able to see? You say you need to be able to see them; what is it that you need to be able to see?

LETBY: We need to be able to see the monitors and the babies themselves.

MYERS: How many years had you been practising as a nurse in neonatal medicine by the time we come to this event in 2015?

LETBY: Three years.

MYERS: How important a part is being able to assess the colour of a baby in neonatal nursing?

LETBY: Oh, hugely. As we see in a lot of the notes, colour is one of the main things that we assess on a regular basis.

MYERS: Where were you when you first noticed, as you describe it, that there was something about [Baby I]'s colour that struck you? Where were you?

LETBY: I was stood in the doorway of the nursery.

MYERS: What was it of [Baby I] that you could see?

LETBY: I could see her face and her hands.

MYERS: And what was it that caught your attention?

LETBY: She just looked very pale.

MYERS: So what happened -- moving on, what happened when you noticed that?

LETBY: I said to Ashleigh that she looked pale and we both went over and I believe we turned the lights up at that point. I don't know whether they were on a dimmer switch but we definitely put more lighting on and went to [Baby I].

MYERS: All right. What did you do once you went over to [Baby I]?

LETBY: We pulled back her blankets and started to take her clothing off. She wasn't breathing properly, she was gasping and looked very, very unwell, so we started Neopuffing immediately and called for help.

MYERS: Had the apnoea alarm gone off?

LETBY: No.

MYERS: Is there any reason you can think of as to why the apnoea alarm wouldn't have gone off in these circumstances?

LETBY: Yes. The apnoea alarms on the unit are programmed to alarm if a baby doesn't breathe for 20 seconds. [Baby I] was gasping and shallow breathing, so I believe she was breathing within that 20-second period, which would explain why the monitor didn't alarm.

MYERS: So did you and Ashleigh together give [Baby I] support at that point?

LETBY: Yes.

MYERS: Were you joined by other people?

LETBY: Yes.

MYERS: How did they come to join you?

LETBY: We called for help.

MYERS: And did you get help?

LETBY: Yes. I remember Nurse Eagles came straightaway.

MYERS: I'd just like to look with your assistance at some of what was said in the interview or the interviews about [Baby I] on this issue so you can comment on it.

The [Baby I] interviews, which would be helpful if we do go to, ladies and gentlemen, are in -- the final interviews are in volume 1 of the interview bundle. Could you have a look for that

as well, Ms Letby? It's the final set of dividers where the first one says "[Initials of Baby I]".

Page 13 behind the [Initials of Baby I] divider, so page 13 of the [Baby I] interviews. It's the last couple of lines on page 12 is probably a better place to start to keep the sense of this.

We're going to look through some of what was said here. At the bottom of the page, the officer interviewing you -- and let's remember this interview is the first -- during the first series of interviews after the first arrest. So this is in July 2018, so a little under 3 years later. The officer says:

"Question: In your notes you have put 'noted to be pale'. Can you elaborate at all on that? Can you expand any observations at all."

And you go over the page, please, Ms Letby, to page 13, and you say, top of the page:

"Answer: Some of it. When we went into the nursery we put the light on -- the lights aren't on in the nursery at night and we put the lights on or something and I noticed -- I looked over at [Baby I] and I noticed that she was pale in colour in the cot."

Do you see that?

LETBY: Yes.

MYERS: You've described to us in your evidence now putting the lights on after you'd first noticed something about [Baby I].

LETBY: Yes.

MYERS: Can you help us with why when you're referring in the interview to the notes and what you're saying there why it is you're saying you put the lights on before you saw her?

LETBY: I can't recall, sitting here now, definitively when the lights did go on --

MYERS: Right.

LETBY: -- but I just know what I could see at that time.

MYERS: What was your initial recollection about when the lights had been put on? Just when you first began to think about this when did you think the lights had been put on?

LETBY: When we went into the nursery.

MYERS: And before or after you has seen [Baby I]?

LETBY: Before.

MYERS: So initially you thought the lights had been put on before you saw [Baby I]?

LETBY: Yes, because Ashleigh was doing something at the workbench.

MYERS: Let's move on then to the third interview where this theme is picked up. So we're moving a couple of dividers forwards, ladies and gentlemen, to an interview that took place now in June 2019, so about a year later, and we are going to go to page 30. Let's take time to get there. Behind divider 3. Keep turning until we come to [Initials of Baby I] 30.

If we start just at the top of page [Initials of Baby I] 30, can you see the officer now is asking about when you saw [Baby I] and the lights?

LETBY: Yes.

MYERS: The officer says:

"Question: Was this the first time you'd noticed [Baby I] was pale, Lucy?"

This is talking about you coming back to the nursery. Within that paragraph, as I've said, I'll just go through it again:

"Answer: When we went into the nursery, put the lights on -- the lights aren't on in the nursery at night and we put the lights on for something and I noticed -- I looked over at [Baby I] and I noticed that she looked pale in colour in the cot."

"Question: Is that accurate? Was that the first time you saw her pale?"

And what was your answer to that?

LETBY: "From my memory, yes."

MYERS: Is that what your recollection was?

LETBY: Yes.

MYERS: If we go down to the bottom of that page, the officer says or asks:

"Question: Am I right in saying that you wouldn't have seen her pale without the lights on? You put the lights on?"

What was your answer to that?

LETBY: "It'd be harder to tell if she was pale with the lights off."

MYERS: And is that so?

LETBY: Yes.

MYERS: Do you agree with that?

LETBY: Yes.

MYERS: Would you be able to tell if she was pale without the lights being turned up?

LETBY: Yes.

MYERS: Right. You're asked:

"Question: So you were with Ashleigh at the time?

"Answer: I think so, yeah. I'm not sure at what point we put the lights on, whether that was before or after we saw [Baby I]."

Is that the state of your recollection at that point?

LETBY: Yes.

MYERS: The officer says:

"Question: You said there, Lucy, 'We put the lights on for something'."

"Answer: Yes.

"Question: And, 'I noticed I looked over at [Baby I]'s --

"Answer: Yeah.

"Question: -- 'and I noticed she looked pale in colour'. [Over the page.]

"Answer: Yeah.

"Question: That would suggest you've noticed having put the lights on.

"Answer: Having put the lights on, yeah."

At this point is that what your recollection was?

LETBY: Yes.

MYERS: Had you assumed any significance yourself, Ms Letby, as to exactly when the lights had been put on or not?

LETBY: No.

MYERS: Did you think there was any reason as to whether or not you would be able to see [Baby I] depending upon when the lights were turned up or put on?

LETBY: Can you say that again?

MYERS: Did you think it made any difference whether or not you'd have been able to see [Baby I]'s colour whether or not the lights were on?

LETBY: I know what I saw (overspeaking) --

MYERS: You know what you saw?

LETBY: Yes.

MYERS: The section ends the page 34 but let's take it through so nothing is missed out. Reading on:

"Question: Do you remember where you would have been stood then over the incubator to see her pale? Would that have been directly over her towards the end?

"Answer: Yeah, I don't -- I think she was in a cot at that point.

"Question: Right okay.

"Answer: It would have been -- she was in the cot space right near to the -- the nursery entrance so it would have been -- once you put the light on it would have been quite easy to see into the cot."

This is you still on the assumption the light was put on; is that right?

LETBY: Yes.

MYERS: "Question: Okay. Is that the reason why you went over with Ashleigh at that time?"

Over the page:

"Answer: I don't remember why."

Do you see that?

LETBY: Yes.

MYERS: And you were asked:

"Question: Do you remember or did you examine [Baby I] prior to switching the light on at all?"

What was your answer to that?

LETBY: "I can't remember."

MYERS: Actually, outside what the notes say, how well can you remember any of this outside the notes?

LETBY: Not very well.

MYERS: How clear was your recollection that you had seen [Baby I] looking pale?

LETBY: Yes, I know I'd seen [Baby I] looking pale.

MYERS: Any doubt about that --

LETBY: No.

MYERS: -- whatever the lights were doing?

LETBY: No.

MYERS: The officer then says:

"Question: Ashleigh Hudson states that [Baby I] was doing well, she'd gone out of nursery 2 for about 15 minutes and when she returned you were stood in the doorway and you looked towards [Baby I] and said that she looked a little pale; do you remember that?

"Answer: No, I remember us both being in the nursery together putting the lights on."

Yes?

LETBY: Yes.

MYERS: That's a recollection you have, is it?

LETBY: Yes.

MYERS: The question before then, "Ashleigh Hudson states", before that point, had you had your attention drawn to what Ashleigh Hudson had said?

LETBY: No.

MYERS: Right. Let's carry on:

"Question: Do you remember telling Ashleigh that [Baby I] looked a little bit pale at the doorway?"

And what was your answer to that?

LETBY: "Yes."

MYERS: And is that something you do remember?

LETBY: Yes.

MYERS: You remember that?

LETBY: Yes.

MYERS: "You do", the officer says. You say, [Initials of Baby I] 34 over the page:

"Answer: I think, yeah, but I think it's when we put the lights on."

And the officer asked this:

"Question: Do you remember exactly the order of events, Lucy?"

How you knew she was pale? And did you remember exactly the order of events?

LETBY: No.

MYERS: What's your answer to "Do you remember exactly the order of events"?

LETBY: No from my -- what do you mean, sorry?

MYERS: Go ahead.

LETBY: "No, from my memory we were both at the nursery, we were both inside the nursery. I think I looked over at [Baby I] -- I

don't know. I thought that we put the lights on as we went into the room."

MYERS: The officer says:

"Question: She says the first time she saw [Baby I] pale -- how could you see from the doorway that [Baby I] was pale without having the light on, Lucy [as read]?"

And your response?

LETBY: "Maybe I spotted something that Ashleigh wasn't able to spot."

MYERS: In fact, can you recall, given what you're saying, who was facing where as between you and Ashleigh in that nursery?

LETBY: Yes, so Ashleigh had her back to the nursery and was stood at the worktop facing the window that faces out of the nursery. I was stood in the doorway looking at Ashleigh and [Baby I] was in my direct eyeline.

MYERS: Right. What do you say at the end of this section?

What do you say next?

LETBY: "The rooms are never that dark that you would not be able to see the baby at all. There is always a level of light for that reason."

MYERS: All right. And it goes on then to talk about the colouring, but pausing with that last answer there:

"The rooms are never that dark that you would not be able to see the baby at all. There's always a level of light for that reason."

However much turned up or down the lights are, I ask you again, would they ever be so dark you could not see the baby?

LETBY: No.

MYERS: And when you were in the doorway, could you see [Baby I]'s skin tone, whatever the level of light?

LETBY: Yes, I could.

MYERS: And what did you notice about it?

LETBY: That she was pale.

MYERS: Could we look at the photograph which was selected by Nurse Hudson? It's at J25368. Do you recall from the evidence, Ms Letby, that there were six photographs that were taken?

LETBY: Yes.

MYERS: These are dealt with in agreed facts 51 and 52 but we don't need to go there immediately, ladies and gentlemen, but if anyone wants to make a reference to it, it is 51 and 52 and the photographs were taken in August 2020.

LETBY: Yes.

MYERS: This is about 5 years later. Do you recall the room being as dark as this appears to be?

LETBY: No.

MYERS: There's no baby in this cot.

LETBY: No.

MYERS: Would you ever have a high dependency nursery with the inside of the cot as dark as this appears to be on this photograph?

LETBY: No.

MYERS: Why not?

LETBY: Because you can't observe the babies in the nursery safely.

MYERS: You described earlier on about who was positioned where with regard to [Baby I].

LETBY: Yes.

MYERS: First of all, is the cot in this photograph in approximately the same position that it was when [Baby I] was in it?

LETBY: No, I believe it was nearer to the workbench (overspeaking) from my recollection because there were three babies in the nursery.

MYERS: So far as the workbench is concerned and the cot, you mentioned it earlier, but now we have the photograph, can you describe to us where Ashleigh Hudson was when she went into the room?

LETBY: So Ashleigh Hudson -- can I use the --

MYERS: Yes, go ahead.

LETBY: Ashleigh Hudson was stood here (indicating) doing something on the worktop with her back to [Baby I]'s cot and therefore facing out through the nursery window.

MYERS: Right. Would she have been looking in [Baby I]'s direction as it happens?

LETBY: No, I can't remember what she was doing but she was focusing on something on the workbench.

MYERS: Where is the door where you were standing, if you can describe this for us, in relation to this photograph?

LETBY: The photograph is taken in my position almost --

MYERS: Right.

LETBY: -- stood within the doorway.

MYERS: That's the position?

LETBY: Well, I can't see exactly, but roughly, yes, yeah.

MYERS: Therefore is that the type of view that you had?

LETBY: Yes.

MYERS: All right, thank you. I want to be quite clear: was it anything like as dark as this on the night that you were looking after Ashleigh (sic) --

LETBY: No.

MYERS: -- or when you went in there?

LETBY: No.

MYERS: Was it necessary to turn the lights up after you saw Ashleigh (sic) --

LETBY: Yes.

MYERS: -- after you saw [Baby I]?

LETBY: Yes.

MYERS: Why did you turn the lights up at that point?

LETBY: At that point we knew [Baby I] needed help and we needed to be able to see more clearly so we did put the main lights on.

MYERS: If a nurse is preparing milk for a feed, what does he or she have to do?

LETBY: They have to measure the milk out using a syringe and decant that into another bottle and then warm the milk.

MYERS: How easy would it be to do that in semi-darkness?

LETBY: I don't think you could. They're very small syringes, so you would need to be able to see the lines on the syringe.

MYERS: All right.

LETBY: And from my memory, [Baby I] was a lot closer to the workbench than this photograph.

MYERS: We know where the window is in relation to this --

LETBY: Yes.

MYERS: -- on the other side of the table, off the photograph to the right. What part of the neonatal unit faces the window on the other side of the corridor?

LETBY: The nursing station.

MYERS: Who goes to the nursing station?

LETBY: All members of staff when they're not doing anything in the nurseries.

MYERS: What sort of view is there, as a rule, from the nursery (sic) station through that window?

LETBY: You have a clear view of that cot space.

MYERS: I'm going to move next then, please -- we can take the photograph down, Mr Murphy, thank you -- to event 3 relating to [Baby I] and, again, this is one which actually splits into two parts, ladies and gentlemen.

Just to remind us, event 3 is 14 October now. So we've just been looking at the 13th at 3.20 in the morning and we are now moving to the 14th, a little bit more than 24 hours later. This event has two parts: the first at 05.00, or thereabouts, and the second at 07.30 or thereabouts, two parts which are being looked at in the evidence.

Are you ready to move on and deal with that, Ms Letby?

LETBY: Yes.

MYERS: Okay, all right. We will come to the notes in a moment but let's just look at the responsibilities for this night, so we need to go to the [Baby I] event 3 sequence, Mr Murphy, and let's start with tile 6 and tile 7 which has the layout. These will come on to the screen. You can put the interviews to one side, Ms Letby, you don't need those now.

Can we go into these? All right. So with event 2, we were looking at what happened on a shift that began at 8 o'clock on the 12th and went through to 8 o'clock on the morning of the 13th. With event 3, the two parts of it, we have a shift that started on the evening of the 13th and goes through to the morning of the 14th.

LETBY: Yes.

MYERS: Just so we can get our heads there. There's a lot of ground to cover.

Again, before we turn to -- let's just have a look at the responsibilities if we could, the layout for the nurseries, please, Mr Murphy, down to tile 7.

[Baby I] is now in nursery 1, Ms Letby?

LETBY: Yes.

MYERS: And you are her designated nurse?

LETBY: Yes.

MYERS: Were you working on your own with [Baby I] in nursery 1?

LETBY: I was [Baby I]'s allocated nurse but Joanne Williams was also in the nursery.

MYERS: Right. The other nurses elsewhere we can see where they are. It's -- a number of the babies' locations are unconfirmed as it happens from the records, it seems, on this shift. Again, if it comes to medications, dextrose, things like that, what would any nurse have to do in terms of where she went or he went to get it?

LETBY: They would have to go into nursery 1 to get those things.

MYERS: Before we go to the notes, do you have any particular recollections of what it was that happened on this shift and [Baby I]'s deteriorations?

LETBY: No, I don't, no.

MYERS: Was there anything that you did to make [Baby I] unwell on that shift --

LETBY: No.

MYERS: -- or any shift for that matter?

LETBY: No.

MYERS: Specifically, do you recall the desaturation that took place at 05.00 beyond what we have in the notes or the --

LETBY: No, I don't, not with memory.

MYERS: And 7.30 in the morning, do you recall anything about that?

LETBY: No.

MYERS: We'll have a look at the notes that deal with this. Can we look, please, first of all, at your notes, which are at tile 40 in sequence 3 for [Baby I]. Go behind the tile into the notes themselves, Mr Murphy, if we could. Right-hand side.

Ms Letby, we've got notes here made on 13 October 2015, 21.49. So in relation to the start of the shift, how far into the shift are you by this point?

LETBY: I would have taken over care at 19.45, so 2 hours.

MYERS: I'm not going to read through the entirety of that because we're still a little way away, in terms of this night, from the events we're looking at. But you've described the condition of [Baby I] at the start of the shift. Can you tell us, if you look down, we can see about 10 lines down it says:

"Aspirate obtained. Abdomen appears full but soft."

Can you see that?

LETBY: Yes.

MYERS: I'll just check we can all see that. What does it say under that?

LETBY: "Some bruising/discolouration evident on sternum and right side of chest, [query] from chest compressions."

MYERS: All right. Now, when you're talking about query from chest compressions, what chest compressions are those?

LETBY: The chest compressions from the previous night that we've just been talking about.

MYERS: Right -- well, in fact, earlier that morning.

LETBY: Well, yes, sorry.

MYERS: So that follows event 2?

LETBY: Yes.

MYERS: So [Baby I] had CPR, did she, at that time?

LETBY: She did.

MYERS: And there was some marking you've identified here on this record?

LETBY: That's right.

MYERS: Where it says:

"Some bruising/discolouration evident on sternum and right of chest."

I don't want you to guess, are you able to assist us with what area of her chest you are talking about, if you indicate on you, bearing in mind she is a very small baby?

LETBY: I couldn't say from memory now, no.

MYERS: Okay. Let's move forwards then to the events we're dealing with. So we have an addendum, if we scroll down at -- on 14 October at 8.43 in the morning. And the first part of that describes her condition but perhaps you could read this note through for us, Ms Letby.

Would you do that, please?

LETBY: "Written in retrospect. Good blood gas at 01.00. No changes to ventilation made. Oxygen 48 to 60%. Temperature elevated. Incubator reduced and within normal range along with other observations. Mean BP maintained on 10 micrograms per kilogram per minute of dobutamine. Minimal ET and oral secretions obtained. Urine output 3.8ml/kg/hour. Bowels opened

at 07.00. Yellow seedy stool. IV fluids as previously, antibiotics given as prescribed. [Baby I] tolerating handling better, tone appears improved. Remaining pale. Abdomen distended but soft. Nil obtained from NG tube."

MYERS: Pause there for a moment. Up to this point, so far as the note assists us, was there any particular matter of concern relating to [Baby I]?

LETBY: No.

MYERS: Can you read what follows from this?

LETBY: "At 05.00 abdomen noted to be more distended and firmer in appearance with area of discolouration spreading on right-hand side. Veins more prominent. Oxygen requirement began to increase, colour became pale. Reviewed by Reg Neame. Ventilation increased to CMV. Rate 70, 30/6. Gradually requiring 100% oxygen. Blood gases poor as charted. Clear air entry. Slightly reduced on left. Chest movement reduced. Nil obtained from NG tube throughout. Continued to decline. Re-intubated at approximately 07.00. Initially responded well. Consultant Jayaram aware and on way to unit. Abdomen firm and distended. Overall colour pale. X-rays carried out."

MYERS: All right. Pause there. We can see if we look on that, resuscitation is commenced and various drugs were given and care was handed over, you say, at the end to Staff Nurse Tomlins?

LETBY: Yes.

MYERS: She's the next nurse to take over. Outside what's on the note there, do you recall this happening?

LETBY: No, I don't, no.

MYERS: That's the event at 05.00 documented there. If we want to see what happens as we move forwards to 07.30, we need to look at the event described by Nurse Tomlins.

Could we put up tile T111, which contains that. It's on the left-hand side.

We can see -- this is Shelley Tomlins -- the note is made at 9.22 on 14 October and it starts from 07.30 to present, so up to 9.22. Do you recall Nurse Tomlins taking over from you that morning? Do you have a memory of that?

LETBY: I recall that she took over care, yes. I can't remember any details.

MYERS: "07.30 to present. Care of [Baby I] taken over for the long day. Arrived on NNU minutes before arrest. [Baby I] had just been re-tubed when desat/brady occurred and full resuscitation was required to bring her back. See medical notes for full details of resus."

It describes [Baby I] being stable on a ventilator, the pressures and the care given at that point.

Just moving down towards the end of this note, after dealing with that, it describes that [Baby I] has passed urine and had a small bowel movement; do you see that?

LETBY: Yes.

MYERS: "Abdomen very large, pale and veiny with umbilical hernia evident. Area of discolouration noted on right side of abdomen."

Do you recall an area of discolouration going forwards to about 7.30? You've referred to it in your notes at 05.00 but anything at 7.30?

LETBY: Yes, I think it remained.

MYERS: One moment.

(Pause)

Do you recall any details of that discolouration?

LETBY: No.

MYERS: So:

"Area of discolouration noted on right side of abdomen. Pancuronium administered at 08.30 as [Baby I] was difficult to ventilate and desaturating."

It then goes on to describe the continuing care for [Baby I]. Were you involved in the continuing care for [Baby I] as required after this event?

LETBY: No.

MYERS: Do you recall whether you stayed on the unit or for any length of time?

LETBY: I don't know what time I left, but I don't believe I was there at 9.28, no.

MYERS: Outside the details we have from the notes, do you have any recollection of this?

LETBY: No.

MYERS: Had you done anything to cause this to happen?

LETBY: No.

MYERS: Would there have been other nurses on the unit for handover at 7.30 in nursery 1?

LETBY: Yes, there would have been day shift and night shift staff members present for all of the babies present in that nursery.

MYERS: Right. That's event 3. I'm going to move, if we may, to event 4. Again, there's two parts to this, ladies and gentlemen. These again involve Ashleigh Hudson with care of [Baby I] and an event that was -- the first part takes place shortly before midnight on the 22nd and there's a second part to this just over an hour later at 01.06.

I'm going to ask, if we could, to go to the [Baby I] sequence number 4, Mr Murphy, and just have a look at the layout here, which is at tile 46 first of all and then tile 47. Just to remind us all, after the events on the 13th and 14 October that we have looked at and described as events 3 and 4, [Baby I] had continued to become unwell and she went towards -- after the events of 15 October to Liverpool Women's Hospital. I think that's correct. Sorry, to Arrowe Park, for further treatment. That's on the 15th after the events on the 13th and 14th. She returns to the Countess of Chester on 17 October. So she had been back for about a week or so, perhaps a little bit less than that, by the time of what we're dealing with on event 4, the 22nd into 23 October.

The designated nurse on this shift is Ashleigh Hudson, Ms Letby. We can see who you were designated for if we look at tile 47, please. Where were you on this particular night?

LETBY: I had two babies, one in nursery 2 and one in nursery 3.

MYERS: We will look at the note that's made by Ashleigh Hudson relating to this. Do you have much by way of independent recollection for what happened on this night?

LETBY: Not of any details specifically, no.

MYERS: Are there any details you can help us with by way of free recall as to your involvement with this when it began?

LETBY: I'm not sure when it began.

MYERS: What about --

LETBY: My memory starts from [Baby I] being resuscitated.

MYERS: Right. Do you recall, in relation to that resuscitation, when it was that you'd first had her condition brought to your attention?

LETBY: Not from my memory now, no.

MYERS: Do you recall at any point going to the nursery that she was in?

LETBY: Yes.

MYERS: She's in nursery 1?

LETBY: Yes.

MYERS: Do you recall why it was you went to nursery 1?

LETBY: I believe Ashleigh alerted me at some point that [Baby I] was unsettled, but I couldn't be specific with when that was.

MYERS: So far as you can recall, had you had any particular involvement with [Baby I] leading up to her being unsettled shortly before midnight?

LETBY: No.

MYERS: Do you remember how [Baby I] was when you first went to that nursery?

LETBY: I remember [Baby I] was unsettled and appeared to be rooting and appeared hungry.

MYERS: Can you give us any more detail as to what you mean by unsettled?

LETBY: Um, she was crying, and she looked very hungry, she was making what we call rooting gestures, so she was sucking on her fingers and sort of lip smacking, which indicates that they're hungry and looking for food. At that point [Baby I] was nil by mouth.

MYERS: Yes, she was. Do you know how long she'd been nil by mouth for?

LETBY: I think for a period of time.

MYERS: You don't know how long precisely?

LETBY: No.

MYERS: All right. Again, don't guess at anything you don't know. I'm going to ask is if we can put up tile 233 -- sorry, not 233, tile 91 from sequence 4 for [Baby I]. Let's have a look at the note. Can we go into that, please? We're going to read through this.

It starts down at the bottom left. I'll read through it, Ms Letby, and then there will be some questions for you, some more detail if you can give it to us.

23 October 2015, 03.39. This is a note by Ashleigh Hudson:

"Written for care taken over at 19.45 hours. Emergency equipment, alarm limits and fluids checked. Nursed in an incubator. Philips monitor in situ. SVIA, observations satisfactory as charted. [Baby I] was unsettled and rooting at start of shift. Settled with dummy and containment holding. Long line removed due to constant occlusions. Neonatal nurse Lucy Letby unable to flush, so paediatric Registrar Rachel Chang informed."

Pausing there, do you have any recollection of being required to flush or asked to assist by flushing a long line?

LETBY: I don't recall that from memory, no, but that's common practice.

MYERS: Is it unusual for a nurse to get assistance from another nurse for a task like that?

LETBY: No, you have to be intensive care trained to access a long line, therefore Ashleigh has asked me to flush it for her, which would be routine procedure.

MYERS: She was not intensive care trained --

LETBY: She wasn't, no.

MYERS: -- at this time? So she would have to find a QIS nurse to do that?

LETBY: That's right, yes.

MYERS: And you were QIS?

LETBY: Yes.

MYERS: So that deals with what happened at that part of the shift. I'm going to move down a little bit. It says three lines up:

"Cannula sited in right foot. TPN given peripherally and lipids stopped until long line could be sited. High pressures via this initially, then reduced on repositioning. [Baby I] tolerated this very well. Sucrose offered but happy with dummy. [Baby I] pink and well perfused at this time, awake and alert."

That's the entry so far as it's made by Nurse Hudson. Before we come to -- there's a gap and then we have 23.57, which is the first part of this incident:

"[Baby I] was very unsettled [query] due to hunger as rooting. Dummy offered and containment holding to no effect. [Baby I] then placed prone as this usually settles her. After repositioning, [Baby I] became quiet, apnoeic and dusky in appearance. With the help of Neonatal Nurse Lucy Letby, I repositioned [Baby I] on to her back and at first applied the Neopuff..."

And it has the settings:

"Neopuff applied due to colour and apnoea. Colour didn't improve and [oxygen] and heart rate began to drop rapidly until oxygen at 47% and heart rate 50. Neopuff increased to 50, then 100% with no effect. At the same time paed's Reg Rachel Chang and paed Reg Bhuvaneshwari Ladde were crash bleeped to the unit. This was 00.00 hours. Cardiac compressions started after confirming Philips monitor trace via auscultation. Dr Gibbs called. [Baby I] intubated and placed on a ventilator whilst awaiting chest X-ray. Prior to ventilation, [Baby I] had become more alert and crying. Abdomen soft and non-distended prior to resus, no change from handover. Please refer to clinical notes for resus information."

And then it refers to "X-ray NAD". Do you know what NAD means there?

LETBY: Yes, "no abnormalities detected", so normal.

MYERS: This is the note by Ashleigh Hudson:

"No abnormalities detected. Decision made by medical team to extubate [Baby I] as quite distressed and working against the ventilator. On extubation, [Baby I] awake and alert and breathing independently. Still rooting. Dummy and sucrose given for comfort."

Right. That's the note made by Ashleigh Hudson. We see at 23.57, if we go back to the top of that, please, a description to -- "[Baby I] was very unsettled".

LETBY: Yes.

MYERS: Did you have any awareness of how [Baby I] had been before this point of the evening?

LETBY: No.

MYERS: Where it says "[question mark] due to hunger as rooting", by rooting is that what you were describing to us earlier?

LETBY: Yes, displaying signs of hunger, yes.

MYERS: In relation to the note that Ashleigh Hudson has made, at what point do you recall becoming involved with [Baby I]?

LETBY: At this point I think Ashleigh called me to help.

MYERS: Right.

LETBY: But that's based on the memory around my notes. I don't have independent memory of that.

MYERS: So after the period when [Baby I] had appeared to be unsettled?

LETBY: Yes.

MYERS: Had you been with [Baby I] in any close period of time to that beforehand?

LETBY: Not that I recall, no.

MYERS: One moment, please.

(Pause)

If we go to the neonatal review, please, can we go to the neonatal review for [Baby I], the fourth [Baby I] event. I have it within exhibit reference KLT35. Can we go to page 3 of 8, please? Let's pick 22.00, so line 26. Can we enlarge the lower half of the table. Looking at this, you're in the blue, Ms Letby.

Who are you caring for?

LETBY: I think I'm in the wrong bit, sorry.

MYERS: I apologise, we're on page 3 of 8.

MR JUSTICE GOSS: It should be the last section of that.

MR MYERS: There we are. The page before that one.

LETBY: I'm on page 3.

MYERS: It should say page 3 of 8 in the bottom corner.

(Pause)

Can you see that now? Is that all right? So we've got it on the screen and it starts just at line 26.

LETBY: Yes, sorry.

MYERS: That's all right. Can you see now?

LETBY: Yes.

MYERS: I am just looking to see from the material we have from the prosecution in the neonatal review whether you have any involvement with [Baby I] within the limits of recollection.

You're in the blue, Ms Letby. Who are you caring for when we look at the first page of the neonatal review from 10 o'clock through to 23.18?

LETBY: A baby by the name of HS.

MYERS: If we go back a page to page 2 of 8, can we see who HS is?

LETBY: It's a baby in nursery 2.

MYERS: Right. A baby you're designated to look after?

LETBY: Yes.

MYERS: Thank you. Go back to 3 of 8, please, Mr Murphy, over the page. No involvement recorded by you with [Baby I] over the period up to 23.18?

LETBY: No.

MYERS: Let's go over the page, please, to page 4 of 8.

If we just look at the first seven or so lines down to 63, taking us up to the event at 23.57. Any involvement by you with [Baby I] in this period?

LETBY: No.

MYERS: Do you recall any involvement?

LETBY: No.

MYERS: Do you recall anything before you were called in after the events as first set out in the note by Ashleigh Hudson?

LETBY: No.

MYERS: When you were called in by Ashleigh Hudson, do you remember what [Baby I]'s condition was at that point?

LETBY: Not from my memory now, no.

MYERS: Do you remember anything about whether or not [Baby I] was upset?

LETBY: I recall going to [Baby I] at some point, yes, and she was upset. I can't recall if it's this specific time.

MYERS: Can we go back, please, to the note made by Ashleigh Hudson at tile 91? Thank you. We're just going to carry on with the second part of this event.

Just scroll down to where it starts at 01.06.

It says:

"[Baby I] became unsettled again. Dummy and sucrose offered with no effect. Slowly became dusky and SaO2 [oxygen] dropped to the 60s. Heart rate 70s. I ran to labour ward theatre to inform Paediatric Registrar Chang. Neonatal Nurse Letby and Senior Neonatal Practitioner Chris Booth maintained airway and applied Neopuff at this time. Compressions started. Dr Gibbs called again to unit. [Baby I] intubated and Neopuff breaths continued with 3:1 compressions. Please refer to clinical notes for full resus information and drugs."

Just looking at what we have there, do you recall where you were between going to help Ashleigh Hudson with [Baby I] after the first part of this event and when we come up to the second event at 01.06, the second part?

LETBY: No.

MYERS: Any recollection at all?

LETBY: No.

MYERS: Do you know whether you'd left nursery 1 in between the two times?

LETBY: In between the two?

MYERS: Don't guess. I'm just asking if you have any --

LETBY: I don't have any recollection at all. I don't know.

MYERS: Were you there during the period that Dr Gibbs and Dr Chang had been dealing with the question of whether or not to keep [Baby I] ventilated?

LETBY: I know there was a discussion, I don't know if I was present for that or whether that's something I heard afterwards.

MYERS: Do you recall what it is that led up to what we see in the note where it says:

"Neonatal Nurse Letby and Senior Neonatal Practitioner Chris Booth maintained airway."

Do you know what, in terms of events, had led up to that?

LETBY: I'm not sure if this is the -- when I remember [Baby I] being unsettled, I'm not sure if it's this event or the previous one, so I can't say now from memory.

MYERS: Do you remember being there with Christopher Booth?

LETBY: Yes.

MYERS: And do you remember doing whatever it means when it refers to the airway being maintained?

LETBY: Yes, and I remember Ashleigh going through to the labour ward.

MYERS: Right. Is that after you and Nurse Booth were present?

LETBY: Yes.

MR MYERS: My Lord, there's a little more detail just to deal with before I come to the conclusion of this event.

MR JUSTICE GOSS: Entirely up to you.

MR MYERS: I wonder if I could stop at this point and deal with that tomorrow because we're near to the end of the period and there's some other exhibits to look at --

MR JUSTICE GOSS: Yes.

MR MYERS: -- and although within this count, it's a different topic to turn to. I'd be grateful if we could. Thank you.

MR JUSTICE GOSS: Right. 10.30 tomorrow morning then, please, ladies and gentlemen. Please remember your responsibilities as jurors, with which you are very, very familiar: no research and no communication.

I paraphrase it to that now. Thank you.

(In the absence of the jury)

MR JUSTICE GOSS: I don't know whether it'll be the same members of staff tomorrow. It will be. Could you just be a little bit later bringing Ms Letby up, rather than having a long time to wait in the courtroom before the court sits? Just 5 minutes or so later. Thank you very much anyway.

(4.10 pm)

(The court adjourned until 10.30 am on Tuesday, 16 May 2023)